NCMHCE question rewrites — triage copy

45 cases · 225 questions · triage copy

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ncmhce-G031 Recurrent Sudden Surges of Fear

Panic Disorder · medium

Q2 · TREATMENT · R2, R3

After three negative cardiac workups, Marcus asks in Session 1 what treatment will actually stop the attacks. Which treatment plan should you recommend first?

was: Which is the MOST appropriate first-line treatment?

KEY **c.** Deliver panic-focused CBT: psychoeducation on the panic cycle plus interoceptive exposure.

near-miss **a.** Begin supportive counseling and relaxation training, deferring exposure work until attacks subside.

novice **b.** Teach breathing retraining as a coping tool to abort each attack, without addressing avoidance.

harmful **d.** Recommend medical leave and daily lorazepam as the primary treatment, with counseling paused for now.

☐ approve _____ re-tier _____ note

Q3 · INTAKE · R1, R3

At intake Marcus is embarrassed and still fears 'something was missed' despite three negative cardiac workups. How are those results MOST useful to your clinical work with him?

was: The three negative cardiac workups are clinically MOST useful for what purpose?

KEY **b.** They anchor psychoeducation that reframes his sensations as intense but harmless arousal.

near-miss **d.** They document that no cardiac disease is present, which supports the differential diagnosis.

novice **a.** They show reassurance-seeking works, so repeated checks can be used to manage his fear.

harmful **c.** They prove his symptoms are imagined, so somatic complaints can be set aside in treatment.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R3, R2

In Session 2 Marcus asks whether he should simply keep away from the highway and crowded stores. What is your best counseling response?

was: Marcus is starting to avoid driving and crowded stores. The BEST response is to:

KEY **d.** Build a collaborative fear hierarchy and start graded in-vivo exposure to driving and stores.

near-miss **b.** Track attack frequency and triggers in a daily log before planning any situational work.

novice **a.** Endorse short-term avoidance of the highway and stores until his attacks fully remit.

harmful **c.** Support a medical leave letter and recommend home confinement until the attacks stop entirely.

☐ approve _____ re-tier _____ note

Q5 · CORE · R1

Marcus worries constantly between episodes, and a colleague suggests generalized anxiety disorder. Which feature of his presentation best supports panic disorder instead?

was: What MOST distinguishes Marcus's presentation from Generalized Anxiety Disorder?

KEY **a.** Discrete unexpected attacks with anticipatory fear of the next one, not pervasive multi-topic worry.

near-miss **b.** Autonomic symptoms such as palpitations and sweating, which are absent in generalized worry.

novice **d.** Duration of under six months, which rules out generalized anxiety and points to panic.

harmful **c.** Presence of avoidance behavior, which by itself establishes panic disorder over other anxiety disorders.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R4, R2

Marcus asks you to back his request that his primary care physician prescribe lorazepam for daily use. How should you proceed?

was: Marcus asks you to support a request for daily lorazepam from his PCP. You should:

KEY **c.** Consult the prescriber, with his consent, about benzodiazepine risks while beginning CBT.

near-miss **a.** Refer him back to the prescriber alone and address the medication question after CBT begins.

novice **b.** Tell him to decide with his physician and keep medication out of the counseling work.

harmful **d.** Send the prescriber a letter endorsing daily lorazepam without discussing this with Marcus.

☐ approve _____ re-tier _____ note

ncmhce-G032 Nightly Drinking After Work

Alcohol Use Disorder, Moderate · medium

Q2 · COUNSELING · R2

In session 2 Dana says, "Maybe I just need to be more disciplined," after describing several failed attempts to cut back. Which counselor response is BEST at this moment?

was: Dana says, "Maybe I just need more discipline." The BEST response uses which approach?

KEY **c.** Reflect her wish to manage it herself and explore the gap between that plan and her repeated failed cutbacks.

near-miss **a.** Affirm her discipline plan and move directly into scheduling relapse-prevention skills for the next two weeks.

novice **b.** Educate her that willpower alone rarely works and list the DSM-5-TR criteria she currently meets.

harmful **d.** Confront the denial firmly, telling her she is an alcoholic who must accept powerlessness today.

☐ approve _____ re-tier _____ note

Q3 · INTAKE · R4, R1

Dana denies morning drinking, shakes, or seizures. Before you finalize her treatment plan, which assessment step is MOST important for her safety?

was: Before setting a treatment plan, the MOST important safety screen is:

KEY **b.** Assess her withdrawal history, including past tremor, seizure, or detox admissions, to gauge medical risk.

near-miss **a.** Assess her family history of alcohol problems to estimate genetic loading and relapse vulnerability.

novice **c.** Assess her readiness to change with a stage-of-change measure to set the pace of weekly sessions.

harmful **d.** Assess her drink counts by self-report and defer medical safety questions until she achieves full abstinence.

☐ approve _____ re-tier _____ note

Q4 · TREATMENT · R3, R1

Dana asks whether she has to "go somewhere" or can do this in weekly sessions. Given moderate AUD with no withdrawal history, which level-of-care recommendation fits?

was: Given moderate AUD with no withdrawal history, the MOST appropriate level of care is:

KEY **d.** Recommend outpatient counseling with drinking monitoring, stepping up care if she fails to stabilize.

near-miss **a.** Recommend an intensive outpatient program three evenings weekly to front-load structure and support.

novice **b.** Recommend medically monitored inpatient detox first, since tolerance signals a need for withdrawal care.

harmful **c.** Recommend she self-monitor at home without scheduled sessions, returning only if her job situation worsens.

☐ approve _____ re-tier _____ note

Q5 · COUNSELING · R2, R1

Dana asks whether she must quit drinking entirely or can learn to control it. Which counselor stance is BEST at this point in treatment?

was: Dana asks if she must abstain or can "learn to control" her drinking. The BEST clinical stance is to:

KEY **a.** Review the evidence on moderation versus abstinence with her and support an informed initial goal she chooses.

near-miss **b.** Defer the goal question to a later session and spend this session mapping her drinking triggers.

novice **c.** Tell her that a moderate alcohol use disorder requires lifetime abstinence as the only acceptable treatment goal.

harmful **d.** Endorse controlled drinking without review, since her insight will guide safe limits on its own.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R5, R4

While discussing evenings at home, Dana mentions she sometimes drives her teenagers home after drinking wine. What should you do FIRST?

was: Dana mentions she sometimes drives after drinking with her teens in the car. You should FIRST:

KEY **c.** Name the risk to her children directly, assess how often it occurs, and build concrete driving safeguards.

near-miss **a.** Note the disclosure in the record and revisit it once her drinking goal has been settled in session.

novice **b.** Report her to the state motor vehicle agency first and postpone any safety discussion with Dana until then.

harmful **d.** Accept her assurance that she drives carefully and return the session focus to her workplace deadlines.

☐ approve _____ re-tier _____ note

ncmhce-G033 Depressed Now, But Productive Spells Before

Bipolar II Disorder · hard

Q2 · CORE · R1, R2

At intake Priya described her past elevated periods only as "a couple of really good stretches." Which assessment task should the counselor complete before making any treatment recommendation?

was: What is the MOST important assessment step before recommending treatment?

KEY **b.** Map the lifetime course of elevated or irritable periods, including collateral reports of observable change.

near-miss **a.** Administer a standardized depression inventory to grade the severity of her current six-week episode.

novice **c.** Screen for a personality disorder that could explain her recurring mood shifts and interpersonal strain.

harmful **d.** Record recurrent major depressive disorder and recommend restarting an antidepressant to lift her mood quickly.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R2, R1

Priya reports that after her previous counselor arranged an antidepressant alone she felt "wired and irritable" for a week. How should the counselor interpret that week?

was: Her prior "wired and irritable" week on an antidepressant alone MOST likely reflects:

KEY **c.** As antidepressant-induced activation, a switch signal that points to bipolar spectrum illness.

near-miss **a.** As an early side-effect reaction to dosing that likely needed a slower titration schedule.

novice **b.** As an anxiety comorbidity emerging once her depressive symptoms began to lift with treatment.

harmful **d.** As proof the medication was working, supporting a dose increase to consolidate her improved energy.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R3

Priya asks the counselor why her depressions keep coming back. Which self-management focus should the counselor teach her first?

was: A core psychoeducation and self-management focus for Priya should include:

KEY **d.** Daily mood charting plus steady sleep, wake, and activity routines to reduce episode triggers.

near-miss **a.** Cognitive restructuring of guilt-laden automatic thoughts to lift her current depressive symptoms.

novice **b.** Behavioral activation scheduling aimed at maximizing output during her high-energy stretches.

harmful **c.** Sleep restriction with progressively earlier waking to override her hypersomnia and low energy.

☐ approve _____ re-tier _____ note

Q5 · CORE · R1

A supervisee reviewing Priya's history asks how the record supports Bipolar II rather than Bipolar I. Which rationale is accurate?

was: What MOST distinguishes Bipolar II from Bipolar I in Priya's case?

KEY **a.** Her elevated periods lasted four to seven days without marked impairment or hospitalization.

near-miss **c.** Her depressive episodes have been more frequent and disabling than her elevated periods.

novice **d.** Her elevated periods lacked psychotic features, which rules out a Bipolar I diagnosis.

harmful **b.** Her mood elevations followed an antidepressant trial, so they count as medication side effects.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2, R3

Given Priya's documented hypomanic episodes and her activation on an antidepressant alone, what is the counselor's most appropriate coordination step now?

was: The MOST appropriate next step in care coordination is to:

KEY **b.** Refer for psychiatric evaluation of mood-stabilizer options and continue bipolar-informed therapy.

near-miss **a.** Continue weekly psychotherapy alone for now and revisit a medication referral if her mood worsens further.

novice **d.** Encourage her to retry an antidepressant alone, since her current presentation is depressive.

harmful **c.** Tell her which mood stabilizer to request and its dosing, then proceed with supportive therapy.

☐ approve _____ re-tier _____ note

ncmhce-G034 Struggling Since the Layoff

Adjustment Disorder, with Mixed Anxiety and Depressed Mood · easy

Q2 · CORE · R1

In Session 1, Tom's low mood and worry are clearly tethered to the layoff. Which rationale BEST supports adjustment disorder rather than major depressive disorder for Tom?

was: What MOST supports Adjustment Disorder over Major Depressive Disorder here?

KEY **b.** His distress followed the layoff and impairs him, yet stops short of full major depressive criteria.

near-miss **a.** His symptoms emerged within three months of an identifiable stressor, which fits the onset requirement.

novice **c.** His appetite is intact and he still enjoys some activities, so a mood disorder can be ruled out here.

harmful **d.** His reaction is a normal response to job loss, so no diagnosis or clinical service is warranted.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R2

Tom was functioning well before the layoff and his symptoms are stressor-bound. What is the MOST appropriate primary intervention to begin with him?

was: The MOST appropriate primary treatment is:

KEY **c.** Provide brief, problem-focused counseling that strengthens coping and mobilizes his support network.

near-miss **a.** Begin a 16-week course of structured cognitive therapy targeting his core beliefs about self-worth.

novice **d.** Refer him for a psychiatric medication evaluation to start an antidepressant before counseling begins.

harmful **b.** Delay any treatment for three months to see whether his symptoms remit once he secures new employment.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R1, R2

In Session 2, after describing some progress, Tom asks whether something is "seriously wrong" with him. What is the BEST counselor response?

was: Tom asks if something is "seriously wrong" with him. The BEST response is to:

KEY **a.** Normalize his reaction as an understandable response to a major loss while validating his real distress.

near-miss **c.** Review the DSM criteria for adjustment disorder with him so he understands the formal basis of the diagnosis.

novice **b.** Reassure him that nothing is wrong and that his symptoms will pass on their own within a few weeks.

harmful **d.** Tell him his symptoms signal a serious mental illness that will likely need long-term psychiatric care.

☐ approve _____ re-tier _____ note

Q5 · CORE · R1

One month later Tom returns reporting pervasive anhedonia, worthlessness beyond the job loss, and hopelessness. What should you do?

was: If, over the next month, Tom develops pervasive anhedonia, worthlessness, and hopelessness, you should:

KEY **d.** Reevaluate the diagnosis, as his presentation may now meet major depressive episode criteria.

near-miss **a.** Increase session frequency and continue the current plan, deferring any diagnostic change for now.

novice **b.** Keep the adjustment disorder diagnosis, since the layoff remains the clear cause of his symptoms.

harmful **c.** Attribute the hopelessness to unemployment stress and proceed without screening for suicide risk.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2, R1

As you set Tom's ongoing plan after the gains he described in Session 2, how should you frame his prognosis and next steps?

was: A realistic prognosis and plan for Tom should emphasize:

KEY **b.** Frame recovery as likely and time-limited, with monitoring and a step-up plan if symptoms worsen.

near-miss **d.** Frame recovery as likely and set a fixed six-session cap, ending care once those sessions are used.

novice **a.** Frame the condition as chronic and recommend indefinite weekly therapy to prevent future relapse.

harmful **c.** Promise Tom full symptom relief once he is rehired and cancel any plan for follow-up monitoring.

☐ approve _____ re-tier _____ note

ncmhce-G035 Adult Depression with Caregiver Stress

Major Depressive Disorder, Single Episode, Moderate · medium

Q2 · TREATMENT · R2, R1

Maria now carries a diagnosis of Major Depressive Disorder, single episode, moderate, with anhedonia, fatigue, and poor concentration affecting her teaching. Which initial treatment plan is MOST appropriate?

was: Which treatment approach is MOST appropriate for moderate MDD?

KEY **c.** Begin CBT with behavioral activation, scheduling graded mastery and pleasure activities weekly.

near-miss **a.** Begin supportive counseling focused on grief about her mother's diagnosis, deferring structured work.

novice **b.** Begin exposure-based anxiety protocol targeting her worry about caregiving before addressing mood.

harmful **d.** Begin advising her on antidepressant dosing and defer psychotherapy until her mood lifts.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R4

In Session 1 Maria says it would be easier if she did not wake up, denies intent or plan, and states she would not act because of her children. What is the BEST next step?

was: Maria reports passive suicidal ideation with strong protective factors. What is the BEST next step?

KEY **b.** Complete a collaborative risk assessment and co-create a written safety plan with her.

near-miss **a.** Document the denial of intent and plan and recheck her risk level at the next weekly session.

novice **d.** Contract for safety verbally and shift the session toward relaxation training for her worry.

harmful **c.** Arrange immediate involuntary hospitalization and notify her husband of the disclosure today.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R2

In Session 2 Maria reports she managed only one walk before fatigue stopped her, called in sick twice, and calls herself a burden to her husband. What is the MOST therapeutic response?

was: Maria struggles to complete her behavioral activation homework. What is the MOST therapeutic response?

KEY **d.** Explore the barriers with her and break the activity into smaller, achievable graded steps.

near-miss **a.** Praise the one completed walk and repeat the same assignment for another week without changes.

novice **b.** Suspend the activity plan and use insight-oriented exploration of her childhood family roles.

harmful **c.** Confront her lack of follow-through as resistance and warn that progress requires compliance.

☐ approve _____ re-tier _____ note

Q5 · ETHICS · R3

Maria asks you to speak with her husband so he understands why she is so tired and withdrawn at home. What should you do FIRST?

was: Maria asks you to speak with her husband about her care. What should you do FIRST?

KEY **a.** Discuss her goals for the contact, obtain written consent, and clarify the scope of disclosure.

near-miss **b.** Discuss her goals for the contact and rely on her verbal permission before phoning her husband today.

novice **d.** Discuss her goals for the contact and invite her husband to the next session as a co-client.

harmful **c.** Discuss her goals later and call her husband now to describe her suicidal thoughts and risk.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2

After three weeks of CBT with behavioral activation, Maria's mood and insomnia are largely unchanged, her weight is still down, and she has missed more work. What is the BEST adjustment?

was: After three weeks of CBT, improvement is limited. What is the BEST adjustment?

KEY **c.** Continue CBT and refer for psychiatric consultation to consider adjunctive medication.

near-miss **a.** Continue CBT unchanged for six more weeks before reassessing her response to treatment.

novice **d.** Continue CBT and add weekly relaxation and breathing training to target her caregiver worry.

harmful **b.** Discontinue CBT and refer her out, since limited gains suggest she is not ready for treatment.

☐ approve _____ re-tier _____ note

ncmhce-G036 Pervasive Worry with Situational Panic

Generalized Anxiety Disorder with Panic Attacks · medium

Q2 · INTAKE · R1

At intake James reports two to three energy drinks daily plus three to four alcoholic drinks several nights a week alongside his year of worry. Which assessment task takes priority in this first session?

was: What is the PRIORITY assessment area at intake?

KEY **c.** Quantify his daily energy drink and nightly alcohol intake to weigh substance-induced anxiety.

near-miss **a.** Map the content of his worry across work, health, finances, and relationships to confirm GAD criteria.

novice **b.** Chart the frequency and triggers of his panic attacks to determine whether panic disorder applies.

harmful **d.** Begin sleep hygiene coaching for his insomnia and defer any substance history to a later session.

☐ approve _____ re-tier _____ note

Q3 · CORE · R4

In session one James says he feels ashamed of needing help and notes his family prized achievement and never feeling good enough. What is the best counselor response?

was: James voices shame about needing help. What is the BEST response?

KEY **a.** Invite James to describe the cultural messages he absorbed about self-reliance and help-seeking.

near-miss **b.** Normalize his shame by noting that many high achievers feel this way and move on to goal setting.

novice **c.** Challenge his belief that needing help is weakness using a thought record on cognitive distortions.

harmful **d.** Interpret his shame as denial of illness and warn him that resisting treatment will worsen his panic.

☐ approve _____ re-tier _____ note

Q4 · TREATMENT · R3, R2

James reports in session two that deep breathing made his anxiety worse because it pulled his attention onto his body. How should you proceed with the somatic component of treatment?

was: Deep breathing "made it worse." What is the BEST approach?

KEY **b.** Explain interoceptive sensitivity and plan graded exposure to the body sensations he fears.

near-miss **a.** Switch to progressive muscle relaxation so he can build calming skills before any exposure work.

novice **c.** Instruct him to practice the breathing twice daily until the discomfort habituates on its own.

harmful **d.** Advise him to avoid all body-focused techniques and rely on distraction whenever panic builds.

☐ approve _____ re-tier _____ note

Q5 · ETHICS · R4

At the end of session two James asks whether you can put him on something for the anxiety. What is the best way to handle his medication request?

was: James asks whether you can prescribe medication. What is the BEST response?

KEY **d.** Clarify that you do not prescribe and offer a referral for a psychiatric medication evaluation.

near-miss **a.** Suggest he wait on medication until he has completed a full course of cognitive behavioral therapy.

novice **b.** Explore what medication means to him and defer the referral question to a later point in treatment.

harmful **c.** Recommend a specific SSRI and dosing range so his primary care doctor can write the prescription.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2

Since his panic attack on the highway James drives only back roads, adding time to every trip. How is this back-road routing best conceptualized for treatment planning?

was: His highway avoidance via back roads is BEST understood as what?

KEY **a.** It is a safety behavior whose short-term relief negatively reinforces avoidance over time.

near-miss **b.** It is an adaptive coping choice that lowers his risk while his driving anxiety settles down.

novice **c.** It is a sign that agoraphobia has developed and should replace the generalized anxiety diagnosis.

harmful **d.** It is a rational precaution that argues for coaching route avoidance as his main panic plan.

☐ approve _____ re-tier _____ note

ncmhce-G037 Combat Veteran with Posttraumatic Stress

Posttraumatic Stress Disorder · hard

Q2 · INTAKE · R2

In session two, Marcus discloses that there are firearms at home, disclosed just after telling you he nearly struck his son when startled. What should the counselor do at this point in the session?

was: He mentions firearms after nearly hitting his son. What is the MOST critical action?

KEY **b.** Assess risk directly and plan temporary off-site storage of his firearms with his wife's involvement.

near-miss **a.** Screen for suicidal and homicidal ideation, and address gun access if he endorses intent.

novice **c.** Refer him to a veterans' anger management group to build control before addressing the household firearms.

harmful **d.** Notify police of the firearms and file a mandatory report of child endangerment the same day.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R3

Marcus's PTSD diagnosis is confirmed and means-safety steps are in place. Which treatment should the counselor recommend as the primary intervention?

was: What is the MOST appropriate evidence-based treatment?

KEY **c.** Begin a trauma-focused protocol such as Prolonged Exposure or Cognitive Processing Therapy.

near-miss **a.** Begin present-centered supportive counseling to build stability before any trauma processing.

novice **d.** Begin generalized relaxation training and sleep hygiene coaching to reduce his arousal symptoms.

harmful **b.** Begin hypnotic recovery of unremembered convoy details to complete the traumatic memory record.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R1, R3

Marcus returns the following week and says he skipped the last appointment because he "couldn't make myself come." How should the counselor respond?

was: He missed a session, saying he "couldn't make myself come." Best response?

KEY **d.** Frame the missed session as avoidance, a PTSD symptom, and problem-solve barriers to attending.

near-miss **b.** Explore his ambivalence about treatment with reflective listening and revisit goals.

novice **a.** Have him sign a behavioral contract with consequences for future missed appointments.

harmful **c.** Discharge him from the trauma clinic for noncompliance and mail him a list of community referrals.

☐ approve _____ re-tier _____ note

Q5 · CORE · R3, R1

Marcus says he should have been the one who died instead of his friend. Which conceptualization of this guilt should guide the counselor's treatment planning?

was: His survivor guilt is BEST understood as what?

KEY **a.** As a trauma-related distorted appraisal within PTSD, targeted directly by CPT cognitive work.

near-miss **c.** As a grief reaction to losing his friend, best managed with bereavement-focused counseling.

novice **b.** As evidence of a co-occurring major depressive episode requiring antidepressant referral.

harmful **d.** As an accurate moral judgment about his own conduct that therapy should respect and leave intact.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R5, R3

Marcus reports drinking six to eight beers nightly with rising irritability since his last session. How should the counselor address this in the treatment plan?

was: How should you address his heavy alcohol use?

KEY **b.** Treat the drinking and PTSD concurrently in one integrated plan with monitoring of use.

near-miss **d.** Refer him to an outpatient alcohol program and hold trauma work until his use declines.

novice **a.** Require 30 days of documented sobriety before starting any trauma-focused sessions.

harmful **c.** Tell him to stop drinking abruptly at home tonight and continue the current session schedule.

☐ approve _____ re-tier _____ note

ncmhce-G038 Emotional Dysregulation and Unstable Relationships

Borderline Personality Disorder · hard

Q2 · TREATMENT · R2, R1

Alexis is now engaged in weekly sessions and reports cutting twice since the emergency-room visit. Which treatment plan is best supported by the evidence for her presentation?

was: What is the STRONGEST evidence-based treatment for BPD?

KEY **c.** Refer Alexis to a structured DBT program with weekly skills training and individual therapy.

near-miss **a.** Offer weekly supportive counseling centered on grieving the breakup and rebuilding her routine.

novice **b.** Start a twelve-session CBT depression protocol targeting her automatic thoughts about rejection.

harmful **d.** Defer psychotherapy and refer for medication management until her mood swings stabilize.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3, R2

Alexis phones between sessions after seeing her ex with another woman and says she may hurt herself if you will not talk. What is the most therapeutic response on that call?

was: When Alexis calls implying self-harm, what is the MOST therapeutic response?

KEY **d.** Validate briefly, assess safety, coach one distress skill, and restate the call limit.

near-miss **a.** Extend the call as long as needed to process the breakup, then schedule an extra session tomorrow.

novice **b.** Reflect empathically on her abandonment fears and explore the childhood roots of the panic.

harmful **c.** Tell her you cannot talk outside sessions, end the call, and address it at the next appointment.

☐ approve _____ re-tier _____ note

Q4 · CORE · R1

In session Alexis calls her ex a "soulmate who turned out evil" and moves between praising you and testing you within the hour. Which mechanism best accounts for this?

was: Her idealization followed by devaluation BEST represents what?

KEY **b.** Splitting: others are seen as all good or all bad, with no integrated middle ground.

near-miss **a.** Projective identification: disowned feelings are induced in the counselor and then acted on.

novice **c.** Rapid cycling: discrete mood episodes alternate and drive her shifting views of other people.

harmful **d.** Antisocial manipulation: her praise and blame are deliberate tactics to control the clinician.

☐ approve _____ re-tier _____ note

Q5 · ETHICS · R2, R3

Alexis asks to start working directly on the childhood sexual abuse while she is still cutting weekly. What is the most appropriate ethical and clinical course of action?

was: Given her trauma history, when should trauma-focused treatment begin?

KEY **a.** Defer trauma processing until self-harm is stabilized and distress-tolerance skills are solid.

near-miss **b.** Begin brief trauma narrative work now while continuing to teach her coping skills in each session.

novice **d.** Redirect her from the abuse history and keep sessions focused only on present relationship issues.

harmful **c.** Start EMDR reprocessing of the abuse at the next session to resolve the source of her instability.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R3, R1

When you name the pattern of the crisis call and the hang-up, Alexis says she will find a new therapist. What is the best response in that moment?

was: Alexis threatens to find a new therapist. What is the BEST response?

KEY **c.** Validate her hurt at the confrontation and link the urge to leave to the abandonment pattern.

near-miss **a.** Provide referral names so she can decide freely, and offer to discuss the choice next session.

novice **b.** Reassure her that you care and promise you will not bring up the pattern again in session.

harmful **d.** Accept the termination, end treatment at the end of the hour, and close her case file.

☐ approve _____ re-tier _____ note

ncmhce-G039 Acute Mania with Impaired Insight

Bipolar I Disorder, Current Episode Manic · hard

Q2 · INTAKE · R1

Robert reports "partying too much" in his twenties, including a DUI and an affair. How should this history be interpreted in his intake formulation?

was: His "partying" in his twenties is MOST relevant because it may indicate what?

KEY **b.** It may be an unrecognized manic episode, indicating a recurrent bipolar course.

near-miss **a.** It may be an unrecognized hypomanic episode, pointing toward a bipolar II course.

novice **d.** It may be substance-induced disinhibition that explains his current elevated mood.

harmful **c.** It may be youthful misconduct that carries no weight in the current diagnostic picture.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R2

Robert insists he is "finally thinking clearly" after spending forty thousand dollars and sleeping three hours a night. What is the most important immediate step in his care?

was: Robert lacks insight. What is the MOST important immediate concern?

KEY **c.** Address safety risks, protect finances, and arrange an urgent psychiatric medication evaluation.

near-miss **a.** Begin psychoeducation on bipolar illness and sleep hygiene while monitoring his spending weekly.

novice **b.** Build insight through motivational interviewing before raising medication or financial safeguards.

harmful **d.** Defer any intervention until Robert accepts he is manic and requests a medication referral.

☐ approve _____ re-tier _____ note

Q4 · ETHICS · R3

Robert's wife phones between sessions to report three nights without sleep, mayoral campaign talk, and sexual comments. How should you handle her call?

was: His wife calls with concerns. How should you handle it?

KEY **d.** Listen to her collateral report, disclose nothing about Robert, and seek his written consent.

near-miss **b.** Decline the call and redirect her to bring concerns to a joint session with Robert present.

novice **a.** Confirm his diagnosis and treatment plan with her, since she brought him in as the reporter

harmful **c.** Share his session content with her and agree to conceal the call so Robert stays in treatment.

☐ approve _____ re-tier _____ note

Q5 · COUNSELING · R2

Robert calls his wife's phone report a "betrayal" and threatens to leave treatment today. What is the best clinical response in this moment?

was: Robert threatens to leave treatment. What is the BEST response?

KEY **a.** Acknowledge his frustration, avoid arguing about insight, and keep the door open to returning.

near-miss **d.** Review the risks of untreated mania and ask him to commit to two more sessions before deciding.

novice **b.** Confront the distortion in his thinking directly so he recognizes the mania and stays engaged.

harmful **c.** Tell him treatment is voluntary, close the file today, and end contact with him and his wife.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2, R1

As Robert's acute mania is stabilized, which element of his history most shapes long-term medication planning?

was: Which history is MOST important for his long-term treatment?

KEY **c.** His two depressive episodes treated with antidepressant monotherapy, which can trigger mania.

near-miss **a.** His decreased need for sleep in the current episode, which guides short-term symptom relief.

novice **d.** His DUI and heavy drinking in his twenties, which suggest a primary alcohol use disorder instead.

harmful **b.** His marital affair and sexually inappropriate remarks, which indicate a personality disorder.

☐ approve _____ re-tier _____ note

ncmhce-G040 Ego-Dystonic Intrusive Harm Thoughts

Obsessive-Compulsive Disorder · medium

Q3 · TREATMENT · R2

Emma is exhausted from checking, searching "signs of a psychopath," and texting her mother hourly. Which treatment should form the foundation of her care plan?

was: What is the FIRST-LINE psychotherapy for OCD?

KEY **b.** Exposure and response prevention, with graded exposure to feared thoughts and dropped rituals.

near-miss **a.** Cognitive therapy targeting her beliefs about responsibility and the meaning of intrusive thoughts

novice **c.** Applied relaxation and breathing retraining to lower arousal whenever the harm thoughts appear.

harmful **d.** Insight-oriented exploration of childhood conflicts thought to underlie her aggressive impulses.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R3

Emma describes earlier periods of contamination fear and fear of being gay before the current harm theme emerged. Which rationale should guide how you explain this pattern to her?

was: Her shifting OCD themes over time suggest what?

KEY **d.** The content of obsessions migrates while the underlying doubt-and-ritual process stays the same.

near-miss **a.** Each theme is a distinct problem best treated in sequence, starting with the most distressing one.

novice **b.** The shifting themes reflect an unstable identity that should be addressed before symptom work.

harmful **c.** The spread of themes signals worsening pathology that warrants inpatient stabilization now.

☐ approve _____ re-tier _____ note

Q5 · CORE · R1

A colleague reviewing Emma's chart wonders whether her thoughts about harming her sister signal an emerging psychotic disorder. Which finding most clearly argues against that?

was: What MOST clearly distinguishes her condition from psychosis?

KEY **a.** She experiences the thoughts as repugnant and inconsistent with who she is, and doubts them.

near-miss **b.** Her thoughts started after an identifiable trigger, a true-crime documentary she watched.

novice **c.** She reports no family history of schizophrenia and has no prior psychiatric hospitalizations.

harmful **d.** Her hours of analyzing and checking show the thoughts are held with delusional conviction.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2, R3

Emma has stopped babysitting her sister "to be safe" and now avoids sharp objects. How should this avoidance be handled in her treatment plan?

was: She stopped babysitting "to be safe." This should be handled how?

KEY **c.** Build a hierarchy and reintroduce babysitting in graded steps without checking or reassurance.

near-miss **a.** Postpone exposure to babysitting until her anxiety has decreased through skills training.

novice **b.** Support the break from babysitting as a healthy boundary while treating her generalized anxiety.

harmful **d.** Advise her family to keep her away from her sister until the harm thoughts fully resolve.

☐ approve _____ re-tier _____ note

Q2 · INTAKE · R2

During the first session Emma repeatedly asks you, "Am I dangerous? Would a normal person have these thoughts?" How should you respond to these requests at intake?

was: Her reassurance-seeking should be addressed by doing what?

KEY **c.** Name reassurance as a compulsion that maintains OCD while validating her distressing doubt.

near-miss **a.** Answer her question honestly this once, then explain that further reassurance will not be provided.

novice **b.** Review evidence of her gentleness with her sister so she can see the fear is not realistic.

harmful **d.** Assess her risk of harming her sister at every session and file a mandated report if doubt persists.

☐ approve _____ re-tier _____ note

ncmhce-G041 Childhood Inattention and Hyperactivity

ADHD, Combined Presentation · easy

Q2 · INTAKE · R1, R2

During the intake, Jayden's father says the behaviors are "just how boys are." What should the counselor do next with this belief?

was: The father's "just how boys are" view should be addressed by doing what?

KEY **b.** Provide the father with psychoeducation on ADHD criteria, cross-setting impairment, and Jayden's specific data.

near-miss **a.** Defer discussion of ADHD until rating scales from the teacher and parents have both been scored.

novice **c.** Explore the father's own childhood behavior problems in depth as the primary clinical focus of the coming sessions.

harmful **d.** Agree that boys mature slowly and reassess Jayden's school functioning at the end of the year.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R2

In session two Jayden's parents say they want to try "natural approaches" before considering medication. What is the counselor's most appropriate response?

was: The parents want "natural approaches." What is the MOST appropriate response?

KEY **c.** Endorse behavioral parent training as first-line and share balanced medication facts for later decisions.

near-miss **a.** Table the medication question entirely and spend the session troubleshooting the parents' inconsistent homework.

novice **b.** Refer the parents to a nutrition-based elimination diet program as the main treatment for Jayden.

harmful **d.** Warn the parents that refusing medication is neglect and document their noncompliance for the school.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R1, R2

When Jayden says, "my brain won't let me," how should the counselor respond in the moment?

was: Jayden says his "brain won't let me." This should be handled how?

KEY **d.** Validate his self-awareness, then link it to concrete skills he can use to feel more capable.

near-miss **a.** Praise the comment and move quickly to teaching classroom organization strategies for his desk.

novice **b.** Correct the statement, explaining that his choices, not his brain, drive his classroom behavior.

harmful **c.** Tell him his brain disorder means outcomes are outside his control and lower all expectations.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2

Given Jayden's age and the mixed follow-through at home, which parent-focused intervention should the counselor prioritize?

was: What is the MOST important parent-focused intervention?

KEY **a.** Coach the parents in clear expectations, praise for target behaviors, and home structure supports.

near-miss **b.** Coach the parents in a daily report card system linking school reports to weekend privileges.

novice **c.** Coach the parents in consistent time-out and privilege removal each time Jayden breaks a rule.

harmful **d.** Coach the parents in withdrawing affection after infractions until Jayden's school reports improve.

☐ approve _____ re-tier _____ note

Q6 · CORE · R1

Jayden reports that he "hates school" and that his teacher "hates him." Which clinical interpretation best fits this statement?

was: Jayden says his teacher "hates him." This MOST likely reflects what?

KEY **c.** Eroded self-esteem after repeated correction and negative feedback about his behavior at school.

near-miss **a.** An accurate account of teacher bias needing classroom observation before any other response.

novice **b.** Emerging oppositional defiant disorder marked by resentful blaming of authority figures.

harmful **d.** A credible disclosure of teacher mistreatment requiring an immediate child abuse report to state authorities.

☐ approve _____ re-tier _____ note

ncmhce-G042 Adolescent Fear of Social Judgment

Social Anxiety Disorder · easy

Q2 · TREATMENT · R2

In session 2, Tyler has completed an exposure hierarchy but says, "I've tried forcing myself before and it made things worse." What is your best next intervention?

was: Tyler is skeptical because "forcing himself" made things worse. Address this by doing what?

KEY **b.** Validate his past attempts and explain how graded collaborative exposure differs from flooding himself.

near-miss **a.** Postpone all exposure work and spend the next several sessions building rapport before revisiting the hierarchy.

novice **c.** Teach relaxation breathing to use during exposures so he can suppress the physical symptoms as they arise.

harmful **d.** Direct him to deliver an unrehearsed presentation to his entire class this week so his fears are disproven quickly.

☐ approve _____ re-tier _____ note

Q3 · CORE · R1, R2

Tyler's parents tell him to "just be more outgoing." How should you conceptualize the effect of this message on his clinical picture?

was: His parents saying "just be outgoing" affects treatment how?

KEY **c.** It compounds his shame about symptoms he cannot control and lowers his willingness to engage in help.

near-miss **a.** It reflects limited understanding of anxiety, though his classroom avoidance is what mainly maintains symptoms.

novice **d.** It is a normal parenting response that has little bearing on his symptom course or treatment engagement.

harmful **b.** It shows his parents are the cause of the disorder, so family conflict should replace his individual work.

☐ approve _____ re-tier _____ note

Q4 · TREATMENT · R2

As you finalize Tyler's treatment plan for social anxiety disorder, which combination of components has the strongest evidence base?

was: What are the MOST effective components for social anxiety?

KEY **d.** Cognitive restructuring of evaluative predictions plus graded exposure, adding social skills practice as needed.

near-miss **a.** Graded exposure alone, delivered without cognitive work, since repeated practice reduces fear on its own.

novice **b.** Relaxation training and supportive counseling to lower arousal before any social situations are attempted.

harmful **c.** Insight-oriented exploration of his early childhood shyness, deferring all exposure until those origins are fully resolved.

☐ approve _____ re-tier _____ note

Q5 · COUNSELING · R1

Tyler says he is certain classmates notice his blushing and conclude he is "weird." Which cognitive formulation best captures this?

was: His fear that others notice his blushing represents what?

KEY **a.** Catastrophic predictions of negative evaluation combined with an inflated sense of being the focus of attention.

near-miss **c.** Self-focused attention on his bodily sensations, absent the predicted judgment that peers will condemn him.

novice **d.** A conditioned physiological response to classrooms that reflects a specific phobia of public speaking.

harmful **b.** A delusional belief that peers actively monitor him, indicating an emerging psychotic process needing psychiatric referral.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3

Tyler is 16 and his parents ask you what he discusses in session. Under what condition is disclosure to them appropriate?

was: Tyler is a minor. When may you break confidentiality with his parents?

KEY **b.** Disclose to his parents when there is risk of harm, suspected abuse, or a legal mandate to report.

near-miss **c.** Disclose to his parents whenever clinically useful for treatment, informing Tyler of the content afterward.

novice **a.** Disclose the content of sessions on his parents' routine request, since they consented to his treatment.

harmful **d.** Disclose nothing to his parents under any circumstance, including disclosed suicidal intent, to protect trust.

☐ approve _____ re-tier _____ note

ncmhce-G043 Binge-Purge Cycle with Body Concerns

Bulimia Nervosa · medium

Q2 · INTAKE · R2, R1

In her first session Stephanie says she wants help to "get control" and lose ten pounds, describing rigid food rules she breaks before bingeing. How should the counselor handle this stated goal?

was: Stephanie wants to "lose ten pounds." Address this by doing what?

KEY **b.** Explore what weight loss means to her and explain how food restriction fuels the binge-purge cycle.

near-miss **a.** Defer any discussion of her weight goal until binge and purge episodes have stopped for several weeks.

novice **c.** Build a reduced-calorie meal plan with her so the ten-pound goal is met through supervised dieting.

harmful **d.** Endorse the weight goal and add a daily weigh-in with a two-hour exercise minimum to curb bingeing.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R2

As Stephanie's treatment plan is written for her three-to-four times weekly binge-purge pattern, which approach carries the strongest research support?

was: What has the STRONGEST evidence for bulimia nervosa?

KEY **c.** Deliver enhanced cognitive behavioral therapy, CBT-E, targeting restriction, binge cues, and purging.

near-miss **b.** Deliver interpersonal psychotherapy focused on role disputes, with eating patterns addressed later.

novice **a.** Deliver supportive counseling with weekly weigh-ins to build insight before addressing eating behavior.

harmful **d.** Deliver appetite-control hypnosis and a fasting protocol to interrupt binges between our sessions.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Stephanie reports dental problems and persistent throat soreness alongside vomiting three to four times weekly. What should the counselor do about these findings?

was: Her dental problems and throat soreness require what?

KEY **d.** Refer her for medical and dental evaluation, including electrolyte labs, for purging complications.

near-miss **b.** Refer her to a dentist for enamel erosion and revisit medical screening if the soreness persists.

novice **a.** Address the throat and dental complaints as anxiety symptoms by teaching relaxation and breathing skills.

harmful **c.** Reassure her the soreness is minor, defer any medical workup, and focus sessions on body image work.

☐ approve _____ re-tier _____ note

Q5 · COUNSELING · R2

In session two Stephanie reports her episodes became daily after a work deadline and asks whether therapy is even working. What is the counselor's best response?

was: After symptoms increase she asks if therapy is working. BEST response?

KEY **a.** Validate her doubt, normalize uneven progress, and examine the deadline stress that spiked episodes.

near-miss **d.** Review her food and symptom logs to show measurable gains before discussing her doubts about therapy.

novice **b.** Interpret her doubt as resistance to change and explore childhood roots of her need for control.

harmful **c.** Agree therapy has stalled, suggest a break from sessions, and revisit treatment in a few months.

☐ approve _____ re-tier _____ note

Q6 · CORE · R3, R2

Stephanie has told her fiancé about the bulimia; he is supportive but "doesn't understand" and thinks she should "just stop." What is the most appropriate step for the counselor?

was: Her fiancé "doesn't understand." What is the MOST helpful step?

KEY **c.** Obtain her written consent, then provide the fiancé psychoeducation about the binge-purge cycle.

near-miss **a.** Coach her in session on how to explain the disorder to him, keeping the work between the two of us.

novice **b.** Invite the fiancé to the next session and orient him then, documenting her verbal agreement today.

harmful **d.** Call the fiancé to explain her symptoms so he can monitor her meals and report purging to me.

☐ approve _____ re-tier _____ note

ncmhce-G044 First-Episode Psychosis in a Young Adult

Schizophrenia · hard

Q2 · TREATMENT · R2, R1

In session two Daniel, now somewhat improved on medication, asks whether he is "going crazy" and if he will ever be normal again. How should the counselor respond?

was: Daniel asks if he will be "normal again." What is the MOST therapeutic response?

KEY **b.** Acknowledge how frightening this is, and share honestly that many people with this illness rebuild meaningful lives.

near-miss **a.** Redirect the question to a coping-skills exercise and revisit his concerns after his symptoms have stabilized further.

novice **c.** Explain the diagnostic criteria for schizophrenia in detail so he can see his experiences are not madness.

harmful **d.** Tell him schizophrenia is a lifelong deteriorating illness and that he should lower his expectations for the future.

☐ approve _____ re-tier _____ note

Q3 · TREATMENT · R2

Daniel's mother asks what services beyond medication her son should receive after this first episode. Which referral should the counselor prioritize?

was: What is the MOST important psychosocial intervention for first-episode psychosis?

KEY **c.** Refer Daniel to a coordinated specialty care team offering case management, family education, and CBT for psychosis.

near-miss **a.** Refer Daniel to a weekly psychoeducation group focused on medication adherence and relapse warning signs.

novice **b.** Refer Daniel to individual insight-oriented psychotherapy aimed at resolving unconscious conflicts that underlie his delusions.

harmful **d.** Refer Daniel to a residential program and advise his mother to limit contact until his symptoms fully remit.

☐ approve _____ re-tier _____ note

Q4 · COUNSELING · R1

During session one Daniel reports voices that comment on him and at times tell him what to do. What should the counselor assess first?

was: He hears commanding voices. What is the PRIORITY assessment?

KEY **a.** Ask Daniel what the voices are telling him to do and whether he has acted on any of those commands.

near-miss **b.** Ask Daniel how often the voices occur and how distressing they are on a scale from one to ten.

novice **c.** Ask Daniel to describe the chip the professor implanted and how it might be sending him these messages.

harmful **d.** Ask Daniel to disregard the voices as symptoms and to stop discussing them so they lose their power.

☐ approve _____ re-tier _____ note

Q5 · ETHICS · R3, R2

After session two Daniel's mother phones the counselor alone and asks directly what her son's long-term prognosis will be. How should the counselor proceed?

was: His mother asks about his prognosis. What is the BEST response?

KEY **d.** Tell her you need Daniel's consent to share details and offer to meet together with him present.

near-miss **a.** Give her general education about first-episode psychosis without discussing Daniel's specific course.

novice **b.** Tell her that confidentiality bars you from discussing any part of his care with family members.

harmful **c.** Share your full prognostic assessment with her since she brought him in and manages his household.

☐ approve _____ re-tier _____ note

Q6 · CORE · R1

Daniel's mother describes an eight-month period of gradual withdrawal, declining hygiene, and school failure before the voices began. How is this period best characterized?

was: The eight-month decline before acute symptoms is BEST described as what?

KEY **b.** The prodromal phase of continuous illness signs that preceded the onset of frank psychotic symptoms.

near-miss **a.** The residual phase in which attenuated symptoms persist after the active psychotic episode resolves.

novice **c.** The onset of a major depressive episode whose withdrawal later evolved into psychotic features.

harmful **d.** The premorbid personality pattern that is unrelated to the current disorder and needs no clinical attention.

☐ approve _____ re-tier _____ note

ncmhce-G045 Faith, Fatigue, and the Weight of Years

Major Depressive Disorder, Moderate · medium

Q2 · COUNSELING · R2, R3

At intake, Walter states he came only because his daughter would not let it go and voices distrust of clinics after past dismissive care. What should the counselor do first to establish a working alliance?

was: What is the most appropriate way to build rapport given his stated mistrust of clinical care?

KEY **c.** Acknowledge the dismissive care he described and ask what he needs from this counselor to feel respected here.

near-miss **a.** Move quickly into symptom rating scales so Walter sees measurable proof that counseling works for him.

novice **b.** Offer general reassurance about confidentiality and move on to the standard intake checklist questions.

harmful **d.** Interpret Walter's distrust of clinics as a paranoid symptom and document it as a barrier to future engagement in care.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3, R2

In session 1, Walter leans forward and insists his pastor and his prayers ought to be enough, questioning whether counseling respects his faith. How should the counselor respond?

was: He says prayer should heal him. How should the counselor respond to his faith?

KEY **b.** Affirm his faith as a strength and present counseling as a complement his pastor's support can reinforce.

near-miss **d.** Explore what prayer has provided him so far, then defer any discussion of counseling's role to later sessions.

novice **a.** Challenge the belief that prayer should suffice as a cognitive distortion and dispute the evidence for it.

harmful **c.** Advise Walter to set aside religious coping until symptoms remit, since it is delaying his recovery.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Walter reports poor concentration, forgetfulness, and slowed movements during intake. Which rationale best supports Major Depressive Disorder rather than a major neurocognitive disorder?

was: Which feature best distinguishes his presentation from a major neurocognitive disorder?

KEY **d.** His attention problems emerged alongside six weeks of low mood and grief, not a gradual decline in memory.

near-miss **b.** His orientation and language remain intact on exam, which makes a neurocognitive process less likely at his age.

novice **a.** His weight loss and early waking are somatic signs that rule out any neurocognitive contribution.

harmful **c.** His age and recent widowhood make his memory complaints an expected feature of typical aging.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

By session 2, Walter agrees to involve his faith community as a support. Which initial treatment approach best fits his symptom profile and stated values?

was: Which initial treatment plan best fits his presentation and values?

KEY **a.** Begin behavioral activation that schedules choir practice and church contacts as graded pleasant activities.

near-miss **c.** Begin weekly supportive counseling focused on grief over his wife, deferring activity scheduling for now.

novice **b.** Begin graded exposure to feared social settings with relaxation training to reduce his avoidance of the choir.

harmful **d.** Begin a medication-only referral and end counseling contact until a psychiatrist evaluates his symptoms.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3

After session 2, Walter's daughter telephones the counselor asking for detailed updates on how her father is progressing. What is the counselor's most ethical action?

was: His daughter requests detailed updates on his progress. What is the most ethical response?

KEY **c.** Ask Walter what, if anything, he wants shared with his daughter and document his written authorization.

near-miss **a.** Invite the daughter into a joint session so any updates are given with Walter present in the room.

novice **b.** Give the daughter general progress updates only, since she brought him in and is his main support.

harmful **d.** Send the daughter detailed session notes, including his passive death wishes, so she can monitor him at home.

☐ approve _____ re-tier _____ note

ncmhce-G046 The Worry That Will Not Rest

Generalized Anxiety Disorder · easy

Q2 · COUNSELING · R2

At intake Rosa's teenage son is relaying her Spanish for her. How should the counselor arrange language access going forward?

was: What is the most appropriate way to handle language access in this case?

KEY **b.** Schedule a trained interpreter for all sessions and provide Spanish-language forms, releasing her son from that role.

near-miss **a.** Ask the bilingual office assistant to interpret today and add Spanish materials once they are translated.

novice **c.** Continue relying on her son to interpret and review the limits of confidentiality with both of them at the start of each session.

harmful **d.** Discharge Rosa with a list of Spanish-speaking agencies and no coordination, citing the language barrier.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

In session 1 Rosa looks down and says it feels selfish to come to counseling for herself. What is the counselor's best response?

was: She feels guilty putting her own needs before family. How should the counselor respond?

KEY **c.** Reframe treatment as building her strength so she can keep caring for the family she values.

near-miss **b.** Normalize the guilt as common and move on to teaching diaphragmatic breathing this session.

novice **a.** Challenge her belief that family must come first, labeling it a distortion to be disputed.

harmful **d.** Advise Rosa to reduce her caregiving duties at home and tell her family she is unavailable for a month.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

In session 2 Rosa reports worry across many domains most days and denies discrete attacks. Which finding best rules out panic disorder?

was: Which feature best distinguishes her presentation from panic disorder?

KEY **d.** Her anxiety is persistent, spread across many domains, rather than sudden peaking surges of terror.

near-miss **b.** Her worry is accompanied by muscle tension and fatigue, symptoms uncommon in brief panic episodes.

novice **a.** Her symptoms began eight months ago, a duration that is too long to meet criteria for panic disorder.

harmful **c.** Her ability to work and care for family shows the anxiety is subclinical rather than a disorder.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R3, R1

Rosa's GAD is established and she asks what the work will involve. Which treatment plan best fits her generalized worry?

was: Which intervention is most appropriate for her generalized worry?

KEY **a.** Deliver cognitive restructuring with worry exposure and relaxation skills she practices daily.

near-miss **b.** Teach progressive muscle relaxation and breathing retraining alone as the complete treatment plan.

novice **c.** Deliver interoceptive exposure to bodily sensations, the protocol used for panic disorder.

harmful **d.** Advise Rosa to avoid situations that trigger worry until her symptoms fully settle down.

☐ approve _____ re-tier _____ note

Q6 · CORE · R1, R2

Rosa states that her family and her faith give her strength and she wants that respected. Which case conceptualization best guides her care?

was: Which best reflects culturally responsive case conceptualization for this client?

KEY **b.** Integrate her familismo and faith as treatment resources while addressing the GAD symptom picture.

near-miss **c.** Document her cultural values in the record and focus sessions on the worry symptoms themselves.

novice **a.** Attribute her worry primarily to acculturative stress and make cultural adjustment the treatment focus.

harmful **d.** Treat her reliance on faith and family as avoidance that maintains worry and should be reduced.

☐ approve _____ re-tier _____ note

ncmhce-G047 Carrying What Was Survived

Posttraumatic Stress Disorder · hard

Q2 · COUNSELING · R3

During intake, Devin asks whether you actually offer affirming care or whether he will have to explain himself the whole time. Which response is best?

was: He asks whether you provide affirming care. What is the best initial response?

KEY **c.** Affirm that you practice affirming care, ask the name and pronouns he uses, and what that care means to him.

near-miss **a.** Reassure him briefly that he is safe here and redirect to gathering the trauma history he came in to discuss.

novice **d.** Explain that you treat every client the same way regardless of identity, then continue the intake questions.

harmful **b.** Tell him identity issues are outside your focus and refer him elsewhere before completing the assessment.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R4, R3

In session 1 Devin's voice flattens as he describes chronic misgendering and prior invalidating providers. How should the counselor handle this alongside his PTSD symptoms?

was: How should the counselor address minority stress alongside his trauma symptoms?

KEY **a.** Treat the assault-related PTSD while framing ongoing misgendering as a stressor that compounds his symptoms.

near-miss **b.** Address the PTSD symptoms first and postpone discussion of minority stress until his arousal has stabilized.

novice **c.** Reframe his reports of misgendering as trauma-driven cognitive distortions to be challenged in session.

harmful **d.** Locate his symptoms in unresolved identity conflict and focus sessions on reconsidering his gender.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Eight months have passed since Devin's assault. Which rationale most clearly separates his presentation from acute stress disorder?

was: Which feature most clearly distinguishes his diagnosis from acute stress disorder?

KEY **d.** His symptom course of eight months extends past the three-day-to-one-month window that defines acute stress.

near-miss **a.** His intrusion, avoidance, and arousal symptom clusters are all present rather than isolated dissociative signs.

novice **b.** His nightmares and startle response are more severe than acute stress disorder typically would produce.

harmful **c.** His exposure to an interpersonal assault rules out acute stress disorder as a possible diagnosis here.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

By the end of session 2 Devin voices cautious relief and asks how the work will proceed. Which treatment plan is best supported for him?

was: Which treatment approach is best supported for his PTSD?

KEY **b.** Offer cognitive processing therapy with collaborative pacing and affirming use of his name and pronouns.

near-miss **a.** Offer supportive counseling and relaxation skills now, deferring trauma-focused work until trust deepens.

novice **c.** Offer insight-oriented exploration of childhood attachment patterns as the main focus of the trauma work.

harmful **d.** Offer prolonged imaginal exposure in the first session at full intensity to reduce avoidance quickly.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R4

After session 2, you recognize that you have little experience with the specific care needs of transgender clients. Which action is most ethical?

was: You realize you lack experience with transgender clients' specific needs. What is most ethical?

KEY **c.** Obtain consultation and affirming-care training while continuing the work Devin has chosen to begin here.

near-miss **b.** Refer Devin to a gender-specialist clinician immediately, framing your limited experience as the reason.

novice **a.** Ask Devin to educate you about transgender identities each session so that you can build competence with him.

harmful **d.** Proceed without consultation and rely on Devin's trust that your general trauma training is sufficient.

☐ approve _____ re-tier _____ note

ncmhce-G048 A Long Way From Home

Alcohol Use Disorder, Severe · medium

Q2 · COUNSELING · R2, R4

At intake Emmanuel sits with arms crossed and minimal eye contact, saying that if people back home knew he was here the shame would follow his whole family. Which counseling stance should the counselor adopt in this first contact?

was: He is reluctant and fears community shame. What is the best counseling stance?

KEY **b.** Explore his ambivalence with open questions while validating the shame and stigma he fears from his community.

near-miss **a.** Provide psychoeducation on alcohol's liver effects first, then invite him to discuss his community concerns.

novice **c.** Confront his denial directly by listing the consequences he minimizes so he accepts the severity of drinking.

harmful **d.** Warn him that treatment cannot proceed unless he first discloses his drinking to his wife and community elders.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

In the second session Emmanuel says quietly that he is not sure he even wants to stop, wavering between resignation and faint hope. How should the counselor respond?

was: He says he is unsure he even wants to change. How should the counselor respond?

KEY **c.** Reflect his mixed feelings and explore both what drinking gives him and what it has cost him.

near-miss **b.** Affirm his strengths and then set a firm quit date so his stated hope translates into action.

novice **a.** Assign a relapse-prevention worksheet on triggers so he can build coping skills before the next session.

harmful **d.** Tell him that until he admits he is an alcoholic, counseling will be paused as he is not ready.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

As the counselor documents Emmanuel's alcohol use disorder, which finding most supports coding a severe rather than a mild specifier?

was: Which finding best supports a severe rather than mild specifier?

KEY **d.** He meets six or more criteria, including tolerance, withdrawal shakes, craving, and use despite liver harm.

near-miss **a.** He drinks heavily every morning, a pattern far exceeding low-risk daily consumption guidelines.

novice **b.** He shows marked acculturative stress and profound isolation, which elevate the severity of his drinking disorder.

harmful **c.** He denies being unable to stop, so criteria are limited and a mild specifier fits his presentation best.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

Emmanuel reports drinking each morning to steady himself and shaking hands when he goes without alcohol. What is the counselor's most appropriate next step?

was: Given probable withdrawal, what is the most appropriate next step?

KEY **a.** Refer him for medical evaluation of withdrawal and decide the level of care with him collaboratively.

near-miss **b.** Schedule weekly outpatient sessions and monitor his shakes with a withdrawal scale at each visit.

novice **c.** Begin relapse-prevention counseling and coping skills training before addressing his physical symptoms.

harmful **d.** Direct him to stop drinking entirely tonight and return in a week to report on his abstinence progress.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R4

The employer who referred Emmanuel calls the counselor asking how his treatment is progressing. What should the counselor do?

was: His employer who referred him requests treatment details. What is most ethical?

KEY **b.** Obtain his written authorization specifying the limited information that may be released to the employer.

near-miss **a.** Tell the employer only that Emmanuel is attending sessions, since attendance is not clinical detail.

novice **c.** Decline any contact with the employer and advise Emmanuel to handle all workplace communication himself.

harmful **d.** Release his diagnosis and session notes to the employer because the workplace referral implies consent to disclose.

☐ approve _____ re-tier _____ note

ncmhce-G049 Storms and Steadiness in a Small Town

Borderline Personality Disorder · hard

Q2 · COUNSELING · R1, R2

In session 1 Brittany says on camera, 'Everyone always leaves eventually, so why would this be different?' while picking at her sleeves. Which counselor stance is most therapeutic at this point?

was: She fears abandonment and tests the relationship early. What stance is most therapeutic?

KEY **b.** Hold a predictable frame and clear limits while validating her fear that people leave, session after session.

near-miss **a.** Explore the childhood origins of her abandonment fears in depth before setting any session ground rules.

novice **c.** Offer extra between-session phone calls and flexible timing to prove to her that you will not leave her.

harmful **d.** Name her testing as manipulation and warn that continued challenges could end the counseling relationship soon.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

Mid-session Brittany's voice rises and she snaps that this telehealth work is pointless and she is done coming. What is your best immediate response?

was: During a heated moment she threatens to quit. What is the best response?

KEY **d.** Stay calm, name her frustration, and invite her to look with you at what just happened between you two.

near-miss **a.** Agree to end the session early and revisit the rupture calmly at the start of the next appointment.

novice **b.** Remind her of the written termination policy and review the intake paperwork she signed with you.

harmful **c.** Accept her decision to quit, document her refusal of care, and close the case without arranging follow-up.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

In session 2 Brittany reports mood swings lasting hours that reset with relationship events and have been present since adolescence. Which rationale best separates her presentation from bipolar II disorder?

was: Which feature best distinguishes her presentation from bipolar II disorder?

KEY **c.** Her mood shifts last hours, react to relationship events, and pair with emptiness and identity confusion.

near-miss **a.** Her impulsive spending and conflict show poor impulse control rather than a distinct manic energy surge.

novice **b.** Her symptoms began in adolescence, which rules out any mood disorder diagnosis at her current age.

harmful **d.** Her self-harm urges confirm a personality disorder and remove the need to screen for hypomanic episodes entirely.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

Brittany states there is no DBT specialist within driving distance and she cannot afford travel. What treatment plan best fits her circumstances?

was: With no local DBT specialist, what is the most appropriate plan?

KEY **a.** Provide structured skills-based therapy by telehealth and seek DBT consultation to build your competence.

near-miss **b.** Place her on the waitlist for the distant DBT program and offer supportive check-ins until a slot opens up.

novice **c.** Deliver open-ended insight-oriented work on her past, deferring skills training until trust is firm.

harmful **d.** Tell her that effective care requires in-person DBT and close the case until she can travel there.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R1

Brittany discloses recurrent self-harm urges during a telehealth session from her home, hours from the nearest hospital. What is your most ethically sound next step?

was: She discloses recurrent self-harm urges over telehealth. What is the most ethical step?

KEY **d.** Build a written safety plan with her and verify local crisis and emergency contacts for her rural county.

near-miss **a.** Complete a detailed risk assessment today and defer building the written safety plan to next session.

novice **b.** Ask her to sign a no-harm contract and keep her current weekly telehealth schedule unchanged for now.

harmful **c.** Notify the county sheriff to conduct a welfare check and suspend telehealth sessions until she is fully stable.

☐ approve _____ re-tier _____ note

ncmhce-G050 Running Hot After Service

Bipolar I Disorder, Current Episode Manic · medium

Q2 · COUNSELING · R2

In Session 1 Tyler paces and states, 'Seeking help is for weak people—there's nothing wrong with me.' Which counselor stance is most likely to sustain engagement at this point?

was: He insists nothing is wrong and resists help. What stance is most effective?

KEY **c.** Engage respectfully, name military stigma about help-seeking, and anchor care in his own goals.

near-miss **a.** Postpone engagement work and spend this session gathering detailed collateral history from his spouse.

novice **b.** Confront his denial with evidence of impairment until he concedes that he is manic and needs care.

harmful **d.** Tell him counseling cannot continue without insight and close the case until he admits he is ill.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R1, R2

In Session 2 Tyler's spouse describes the week of change as frightening and uncharacteristic while he interjects and minimizes. How should the counselor respond to her?

was: His spouse is frightened and overwhelmed. How should the counselor respond?

KEY **d.** Validate the spouse's fear, explain manic episode features, and outline the next clinical steps.

near-miss **b.** Refer the spouse to a caregiver support group and revisit her concerns at a later joint session.

novice **a.** Redirect the spouse to focus on her own anxiety symptoms and schedule her for individual counseling.

harmful **c.** Reassure the spouse that the mood swings will settle on their own and discourage further worry.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Tyler reports distractibility, restlessness, and impulsive spending. Which finding best distinguishes his manic episode from ADHD?

was: Which feature most clearly supports a manic episode over ADHD?

KEY **b.** His distractibility emerged in a distinct week-long shift with decreased sleep need and grandiosity.

near-miss **a.** His pressured speech and rapid tangential thinking are prominent during the current interview.

novice **d.** His impulsive spending and motor restlessness reflect a lifelong pattern of inattention and hyperactivity.

harmful **c.** His irritability and denial of illness point to an antisocial personality pattern, not mania.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R1

Given Tyler's week of minimal sleep, escalating spending, and grandiose business plans, what is the counselor's most appropriate immediate treatment priority?

was: What is the most appropriate immediate treatment priority?

KEY **a.** Arrange urgent psychiatric evaluation for medication and assess risk of harm and impulsivity.

near-miss **c.** Begin weekly IPSRT sessions to stabilize his sleep and daily routines before any referral.

novice **d.** Start trauma-focused processing of his deployment memories to reduce his current agitation and anger.

harmful **b.** Support his plan to defer psychiatric care and monitor his mood in weekly counseling alone.

☐ approve _____ re-tier _____ note

Q6 · CORE · R2, R1

Tyler equates treatment with weakness while his spouse pushes for answers. Which approach reflects the soundest clinical reasoning about disclosing his diagnosis?

was: Which best reflects sound clinical reasoning about his diagnosis and stigma?

KEY **c.** Name the bipolar diagnosis clearly and compassionately, treating stigma as part of engagement.

near-miss **a.** Delay naming the diagnosis until his mania remits, describing the symptoms as stress reactions.

novice **b.** Emphasize the chronic, lifelong nature of the illness so he grasps how serious his condition is.

harmful **d.** Withhold the diagnosis from Tyler and share it only with his spouse to avoid provoking him.

☐ approve _____ re-tier _____ note

ncmhce-G051 When the Body Speaks for the Mind

Schizophrenia · hard

Q2 · COUNSELING · R3, R2

In session 1, Kevin's parents keep steering the interview toward his headaches and stomach pain while he describes voices. How should you position your language with this family?

was: The family frames his distress as physical illness. What stance is most helpful?

KEY **c.** Use the family's language of headaches and stomach pain while widening the talk to Kevin's voices.

near-miss **a.** Accept the somatic focus for now and defer any mention of Kevin's voices until several sessions have passed.

novice **d.** Correct the parents' physical framing by explaining that Kevin's pains are symptoms of a psychotic illness.

harmful **b.** Confront the family that their somatic framing is denial and warn that shame will worsen Kevin's psychosis.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

Kevin murmurs that he can hear the voices right now and that neighbors watch him through something placed in his body. What is your best in-session response?

was: He reports voices and believes neighbors monitor him. How should you respond?

KEY **b.** Acknowledge how frightening the voices feel and explore their impact without agreeing or arguing.

near-miss **d.** Redirect Kevin to coping skills for the voices before exploring what the experiences mean to him.

novice **a.** Test the persecutory belief with reality checks, asking Kevin for evidence that neighbors watch him.

harmful **c.** Affirm that the neighbors may be monitoring him and offer to help him find the implanted device.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

In session 2, the history confirms the timeline of Kevin's illness. Which feature most clearly supports schizophrenia rather than schizophreniform disorder?

was: Which feature best distinguishes his diagnosis from schizophreniform disorder?

KEY **d.** His psychotic symptoms have run eight months, passing the six-month limit for schizophreniform.

near-miss **a.** His negative symptoms and flat affect are prominent, which is uncommon in schizophreniform presentations.

novice **c.** His auditory hallucinations and persecutory delusions are severe, which rules out schizophreniform.

harmful **b.** His functional decline is marked, so the picture is best explained by a brief psychotic disorder.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

With schizophrenia established and Kevin untreated for eight months, what treatment plan should you put in place with him and his parents?

was: What is the most appropriate treatment approach for him?

KEY **a.** Refer for psychiatric evaluation and coordinate team care with culturally attuned family psychoeducation.

near-miss **b.** Begin weekly supportive counseling alone and revisit medication referral if symptoms persist next month.

novice **c.** Start trauma-focused therapy for the somatic complaints and defer any psychiatric medication consult.

harmful **d.** Exclude the parents from all care planning and provide individual insight therapy that disputes his beliefs.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R2, R3

After session 2, Kevin's parents ask you privately not to tell him the diagnosis so the household is not shamed. What is your most ethical course of action?

was: The parents ask you to keep the diagnosis secret to protect family honor. What is most ethical?

KEY **b.** Tell Kevin his diagnosis directly while exploring with parents what disclosure means for family honor.

near-miss **c.** Delay naming the diagnosis until the family's shame concerns are fully processed in later sessions.

novice **a.** Explain the diagnosis to the parents first and let them decide when Kevin should be told about it.

harmful **d.** Withhold the diagnosis from Kevin and document that his parents consented to the treatment on his behalf.

☐ approve _____ re-tier _____ note

ncmhce-G052 Rituals, Roots, and Relief

Obsessive-Compulsive Disorder · medium

Q2 · COUNSELING · R3, R2

In Session 2, Aiyana asks whether this treatment can live alongside her community's healing rather than replace it. What is the counselor's best response?

was: She worries treatment may conflict with her ceremonies. What is the best response?

KEY **b.** Ask how ceremony sustains her and frame exposure work as compatible with her traditional healing.

near-miss **a.** Assure her that treatment plans can be revisited later and begin symptom monitoring this session.

novice **c.** Reassure her that ceremony is separate from treatment and focus sessions only on washing behavior.

harmful **d.** Advise her to suspend ceremonial practice during treatment so ritual behavior is not reinforced.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R1

Aiyana glances at her raw hands and says she feels dirty and weak for spending hours washing. Which counselor response is most appropriate?

was: She feels ashamed of her washing rituals. How should the counselor respond?

KEY **c.** Explain the obsession-compulsion cycle in plain terms and normalize her symptoms to reduce self-blame.

near-miss **a.** Reflect her shame with empathy and revisit psychoeducation after rapport has deepened further.

novice **d.** Dispute her belief that her hands are contaminated and argue the objective evidence against danger.

harmful **b.** Agree that her washing is shameful and direct her to halt all handwashing completely before the next session.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

During intake Aiyana describes unwanted contamination thoughts followed by hours of washing; which finding best distinguishes her presentation from generalized anxiety disorder?

was: Which feature best distinguishes OCD from generalized anxiety disorder here?

KEY **d.** Her intrusive contamination thoughts are neutralized by ritualized washing performed to relieve anxiety.

near-miss **a.** Her worry is focused on a single theme rather than spread across many daily life concerns.

novice **b.** Her distress is severe enough to interfere with her work and family functioning most days.

harmful **c.** Her symptoms follow historical trauma, so cultural stress rather than a disorder explains her hours of washing.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

Aiyana reports she resists the washing rituals but cannot stop, and wants a plan that honors her ceremonies. Which intervention is best supported for her OCD?

was: Which intervention is best supported for her OCD?

KEY **a.** Introduce graded exposure with response prevention, pacing hierarchy steps collaboratively with her.

near-miss **c.** Begin cognitive restructuring of her contamination beliefs before any exposure exercises are attempted.

novice **b.** Provide relaxation training and thought stopping to interrupt the intrusive contamination thoughts.

harmful **d.** Direct unplanned flooding by having her touch contaminated surfaces without preparation or consent.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3

Aiyana asks the counselor to speak with the traditional healer who leads her ceremonies about her care. What is the most ethical course of action?

was: She asks you to consult with her traditional healer. What is most ethical?

KEY **b.** Obtain her written release and coordinate care with the healer within agreed limits of disclosure.

near-miss **a.** Encourage her to relay treatment information to the healer herself while records stay confidential.

novice **d.** Decline the request and explain that clinical care and traditional healing must stay separate.

harmful **c.** Contact the healer promptly to discuss her case before paperwork is completed at the next visit.

☐ approve _____ re-tier _____ note

ncmhce-G053 Focus, Function, and Fair Access

Attention-Deficit/Hyperactivity Disorder, Combined · easy

Q2 · COUNSELING · R2, R3

At intake, Marcus scans the room for clearance and asks directly whether this process is actually accessible to him. What is the best initial response?

was: He asks whether the process is accessible. What is the best initial response?

KEY **b.** Ask Marcus what access needs he has and plan accommodations with him before starting the evaluation.

near-miss **a.** Assure Marcus that the office is wheelchair accessible and move directly into the presenting problem.

novice **c.** Give Marcus the standard intake packet and address any access barriers if they come up later.

harmful **d.** Refer Marcus out to a disability specialty clinic, since his mobility needs exceed your competence.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

In session one, Marcus says every clinician before you stared at the chair and skipped past his actual problem. How should you respond?

was: He is frustrated that past clinicians overlooked his attention concerns. How should you respond?

KEY **d.** Acknowledge his frustration and state that this evaluation will center his attention symptoms.

near-miss **a.** Reflect his frustration briefly, then continue the structured symptom checklist as planned.

novice **b.** Explore how his anger toward former providers may reflect unresolved family-of-origin conflict.

harmful **c.** Attribute his frustration to ADHD impulsivity and advise him to avoid criticizing past providers.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Marcus's session-two history, which feature most supports ADHD rather than a specific learning disorder?

was: Which feature most supports ADHD over a specific learning disorder?

KEY **a.** Onset in childhood with inattention and hyperactivity impairing him at both home and work.

near-miss **c.** Reports of losing notes and missing deadlines that worsened after he entered the workforce.

novice **b.** Difficulty completing written reports accurately despite adequate effort and instruction.

harmful **d.** Rapid, tangential speech in session that best indicates an underlying bipolar mood disorder.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

Marcus's evaluation confirms ADHD, combined presentation, with impairment at work and home. Which treatment plan best fits his presentation?

was: Which treatment plan best fits his presentation?

KEY **c.** Skills coaching for organization and attention, plus referral for medication and job accommodations.

near-miss **a.** Skills coaching for organization and attention alone, deferring any medication referral for a year.

novice **b.** Insight-oriented therapy exploring how disability-related stigma produced his attention difficulties.

harmful **d.** Stimulant dosing guidance provided in session, with accommodations deferred until symptoms remit.

☐ approve _____ re-tier _____ note

Q6 · TREATMENT · R2, R3

In session two, Marcus asks for written summaries because he keeps losing track of his own notes. What is the best response?

was: He requests written summaries due to attention difficulties. What is the best response?

KEY **b.** Provide brief written session summaries as an individualized accommodation and review their fit.

near-miss **c.** Teach Marcus a note-taking system in session so he can create his own written summaries.

novice **d.** Explain that written summaries fall outside your standard practice and suggest he record the sessions.

harmful **a.** Decline the request, noting that accommodating attention deficits reinforces his dependence.

☐ approve _____ re-tier _____ note

ncmhce-G054 Devotion, Discipline, and the Body

Anorexia Nervosa, Restricting Type · medium

Q2 · COUNSELING · R3, R1

In Session 1, Sarah says serenely, 'Restraint brings me closer to God—there's nothing wrong here.' Which counseling stance best fits this moment?

was: She frames restriction as spiritual discipline. What is the best counseling stance?

KEY **a.** Honor the meaning her faith gives to fasting while gently naming the physical risks her doctor found.

near-miss **b.** Defer discussion of her beliefs and focus first on charting weight, meals, and vital signs each week.

novice **c.** Challenge her theology directly, showing that her scripture does not require the fasting she practices.

harmful **d.** Affirm the restriction as valid spiritual practice and postpone any focus on her weight or nutrition.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

Sarah states evenly that her physician is overreacting and that she has no disorder. How should you respond in this first session?

was: She denies any problem despite her physician's concern. How should you respond?

KEY **c.** Explore what she values most and any worries she does hold, using reflective, nonjudgmental dialogue.

near-miss **a.** Provide detailed psychoeducation on the medical effects of starvation and ask her to review it at home.

novice **b.** Confront the denial with her physician's lab findings until she concedes that she has a disorder.

harmful **d.** Tell her that treatment cannot proceed unless she first agrees she is ill, and close the case now.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

At intake, Sarah reports fasting, weight loss, and religious rationale for her eating. Which feature best separates her presentation from avoidant/restrictive food intake disorder?

was: Which feature best distinguishes her presentation from avoidant/restrictive food intake disorder?

KEY **b.** Her restricting is driven by intense fear of weight gain and a distorted view of her body's size.

near-miss **a.** Her intake has dropped enough to produce low weight and the medical findings her physician noted.

novice **c.** Her food rules are organized around religious observance rather than sensory or texture aversion.

harmful **d.** Her firm denial of any illness shows the restriction is a faith practice and not a clinical condition.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

In Session 2, Sarah says she would accept help that respects her faith and includes trusted mentors. Which treatment plan best fits?

was: What is the most appropriate treatment approach for her?

KEY **d.** Coordinate medical, nutritional, and therapy care, inviting her trusted spiritual mentors to support.

near-miss **a.** Begin weekly individual therapy on body image and defer medical and dietary referrals for later.

novice **b.** Start graded exposure work on feared foods alone, without dietitian input or medical monitoring in place.

harmful **c.** Refer her solely to her congregation's leaders for guidance and pause the physician's involvement.

☐ approve _____ re-tier _____ note

Q6 · CORE · R3, R1

As you write your case conceptualization for Sarah, which formulation is most culturally responsive and clinically sound?

was: Which best reflects culturally responsive conceptualization of her case?

KEY **a.** Keep the anorexia diagnosis accurate while drawing on her faith and mentors as resources in recovery.

near-miss **b.** Hold the diagnosis firmly and address her religious context only if it obstructs her treatment plan.

novice **c.** Treat the restriction mainly as a cultural variation of fasting and monitor her symptoms over time.

harmful **d.** View her religious practice as the root cause of the illness and steer her away from her faith community.

☐ approve _____ re-tier _____ note

ncmhce-G055 Holding On Through the Night

Suicidal Behavior / Acute Risk · hard

Q2 · COUNSELING · R4, R3

In session 1, Eleanor lifts her gaze and asks, "Do you really understand a woman like me, my community, my faith?" How should the counselor respond?

was: She voices distrust and asks if you understand her community. What is the best response?

KEY **b.** Name her caution about counseling as reasonable and ask how her faith and church family sustain her.

near-miss **a.** Reflect her grief warmly and return to the risk questions, deferring her question about trust for later.

novice **d.** Assure her that counseling works the same for everyone and that her background will not affect care.

harmful **c.** Tell her that faith talk delays treatment and insist she set church supports aside during sessions.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R1, R3

At intake Eleanor says softly, "I've been wondering why the Lord hasn't taken me too." What is the most therapeutic counselor response?

was: She discloses thoughts of not wanting to live. What is the most therapeutic response?

KEY **c.** Reflect the weight of her loss calmly, then ask directly whether she has thought of ending her life.

near-miss **a.** Reflect her sorrow and wait for her to elaborate on the statement before raising the topic of suicide.

novice **d.** Reframe the remark as normal grief and offer psychoeducation on stages of bereavement in older adults.

harmful **b.** Redirect her from the remark, praise her faith, and move on to planning the next scheduled appointment.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

In session 2 Eleanor appears steadier and agrees to a formal review of her suicidal thoughts. Which assessment approach best establishes her current risk level?

was: Which approach best assesses her current level of suicide risk?

KEY **a.** Administer a validated screener rating ideation, intent, plan, past attempts, and protective factors.

near-miss **c.** Ask open-ended questions about her grief and note any risk themes that surface in her own narrative.

novice **d.** Administer a depression symptom inventory and gauge her suicide risk from the total severity score.

harmful **b.** Accept her report that faith fully prevents self-harm and document low risk with no further questions.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2

Eleanor endorses ideation with some intent but no specific plan and names her pastor and grandchildren as reasons for living. What is the safest next intervention?

was: Given her ideation with intent but no plan, what is the safest next step?

KEY **d.** Build a written safety plan with her that lists warning signs, coping steps, her pastor, and crisis lines.

near-miss **b.** Schedule twice-weekly sessions and a bereavement group referral, deferring safety planning until risk rises.

novice **c.** Have her sign a no-suicide contract promising to call the office before acting on any urge to die.

harmful **a.** Send her home with a crisis hotline number and reassess her suicide risk at the next monthly visit.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R2

As safety planning begins, Eleanor asks the counselor to keep her suicidal thoughts from her adult children. What is the most ethical course of action?

was: She asks you not to tell her family about her suicidal thoughts. What is most ethical?

KEY **c.** Review the limits of confidentiality and decide together which supports may be told for her safety.

near-miss **a.** Honor her request for now and revisit family involvement once the therapeutic alliance feels stronger.

novice **b.** Promise complete confidentiality to protect the alliance and document her refusal of family contact.

harmful **d.** Contact her adult children today without telling her, since imminent risk overrides her preferences.

☐ approve _____ re-tier _____ note

ncmhce-G056 Long-Standing Low Mood in a Spanish-Dominant Client

Persistent Depressive Disorder · medium

Q2 · COUNSELING · R2

In the first session Rosa keeps her eyes down, says 'no quiero molestar a nadie con esto,' and describes putting her family's needs before her own. Which approach best builds rapport with her at this point?

was: How should the clinician build rapport with Rosa in the first session?

KEY **c.** Name her family caretaking as valued, validate that role, and follow her pace in exploring her own distress.

near-miss **a.** Focus first on completing symptom rating scales, then explore her family role in a later session.

novice **b.** Encourage her to assert her own needs over family duties as the first rapport-building task.

harmful **d.** Interpret her guarded manner as resistance and press her to disclose the distress she appears to be minimizing.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R1, R2

When Rosa describes her low mood she shrugs and repeats 'asi soy yo, asi he sido siempre.' How should the counselor respond to this statement?

was: Rosa says her sadness is just who she is. What is the best response?

KEY **d.** Reflect how long the mood has lasted, gently separate it from who she is, and invite joint exploration.

near-miss **a.** Accept her self-description for now and revisit the identity question after a few months of therapy work.

novice **b.** Dispute her statement with thought records showing that her belief about herself is distorted.

harmful **c.** Agree that this is her fixed temperament and shift the focus to helping her tolerate it better.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Rosa reports that in over four years her mood has never lifted for more than a few weeks at a time. Which finding most supports persistent depressive disorder rather than major depressive disorder?

was: Which feature most supports persistent depressive disorder over major depression?

KEY **b.** Depressed mood on most days for over four years with no remission lasting more than two months.

near-miss **a.** Presence of low energy, poor self-esteem, and concentration problems alongside her reported low mood.

novice **d.** Her flat affect, slowed speech, and minimal eye contact observed across the two intake sessions.

harmful **c.** Her report of no clear onset, which rules out a depressive disorder and indicates a trait pattern.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R2

Rosa prefers to speak Spanish, reports chronic fatigue and hopelessness, and organizes her life around family obligations. Which initial treatment plan best fits her?

was: What initial treatment approach best fits Rosa?

KEY **a.** Begin CBT with behavioral activation conducted in Spanish and framed around her family values.

near-miss **c.** Begin supportive counseling in Spanish and defer any structured skills work until rapport is firm.

novice **d.** Begin exposure-based work in Spanish targeting her avoidance of discussing distress with family.

harmful **b.** Begin conjoint family sessions in English so relatives can report on her mood and daily functioning.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R2

Rosa states she prefers sessions in Spanish and speaks more fluidly in it, while the counselor speaks only limited Spanish. What does the counselor's ethical obligation require?

was: Rosa is Spanish-dominant and the clinician speaks limited Spanish. What is required?

KEY **c.** Arrange a qualified interpreter or refer her to a Spanish-speaking counselor for ongoing care.

near-miss **a.** Proceed in limited Spanish now and document a plan to obtain interpreter support at a later point.

novice **d.** Conduct sessions in English and provide translated handouts so she can follow the material at home.

harmful **b.** Ask her adult daughter to interpret each session and sign the consent forms on Rosa's behalf.

☐ approve _____ re-tier _____ note

ncmhce-G057 Intense Fear of Judgment in a Non-Binary Client

Social Anxiety Disorder · medium

Q2 · COUNSELING · R3, R1

In Session 1, Jordan's voice drops and hands tremble while describing vigilance about how others read their gender presentation. What should the counselor do at this moment?

was: How should the clinician respond to Jordan's fear of judgment in session?

KEY **b.** Name the fear of scrutiny, affirm Jordan's identity, and make the room explicitly safe to disclose.

near-miss **a.** Move directly into a symptom checklist for anxiety severity before exploring Jordan's fear of judgment.

novice **c.** Reassure Jordan that no one is judging them and redirect the hour toward relaxation breathing practice.

harmful **d.** Label the fear as an irrational distortion and assign an unrehearsed class presentation for next week.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3, R1

In Session 2, after noting the fear existed before they had words for who they are, Jordan asks whether this anxiety is simply the result of being non-binary. How should the counselor reply?

was: Jordan asks whether their anxiety is just because they are non-binary. Best reply?

KEY **c.** Explain that minority stress adds real burden while the anxiety is a distinct, treatable condition.

near-miss **a.** Explain that the cause matters less than the plan, and move straight to setting graded exposure goals.

novice **b.** Explain that the anxiety is fully accounted for by living as non-binary in an unsupportive climate.

harmful **d.** Explain that gender identity is unrelated here and that discussing it would derail the treatment.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Jordan's intake and Session 1 reports of blushing, racing heart, and dread before speaking, which finding best separates this presentation from panic disorder?

was: Which finding best distinguishes social anxiety disorder from panic disorder here?

KEY **d.** Jordan's anxiety surges are cued by situations involving scrutiny, not unexpected attacks.

near-miss **b.** Jordan avoids classes and meetings, an avoidance pattern also seen in panic-related agoraphobia.

novice **a.** Jordan's blushing and racing heart point toward recurrent unexpected panic attacks as the core issue.

harmful **c.** Jordan's fear has lasted over six months, which rules out panic disorder and confirms this diagnosis.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

Given Jordan's six-month pattern of avoiding classes and meetings with intense fear of messing up, which treatment should the counselor initiate first?

was: What is the recommended first-line treatment for Jordan?

KEY **a.** Begin affirming CBT that pairs graded social exposure with cognitive restructuring of beliefs.

near-miss **b.** Begin with an SSRI referral and supportive counseling to reduce Jordan's anticipatory dread first.

novice **c.** Begin applied relaxation and breathing retraining as the primary intervention for the arousal symptoms.

harmful **d.** Begin intensive flooding by requiring that Jordan present unprepared to a large lecture class.

☐ approve _____ re-tier _____ note

Q6 · CORE · R3

At Jordan's first intake appointment, which counselor practice best honors their identity from the outset?

was: Which intake practice best honors Jordan's identity from the start?

KEY **b.** Ask Jordan directly for their pronouns and name, then use them in session and in the chart.

near-miss **c.** Wait for Jordan to volunteer pronoun information so the intake does not feel intrusive.

novice **a.** Copy the gender marker from Jordan's insurance record and mirror whatever wording Jordan uses.

harmful **d.** Ask Jordan detailed questions about anatomy and surgical history to finish the intake form.

☐ approve _____ re-tier _____ note

ncmhce-G058 Recent Overwhelming Stress After a Frightening Event

Acute Stress Disorder · easy

Q2 · COUNSELING · R2, R3

In Session 1 Maria lowers her voice, keeps her arms crossed, and says she does not know whom she can trust here. Which counselor action best establishes a working alliance at this point?

was: How should the clinician approach Maria given her institutional mistrust?

KEY **b.** Name the limits of confidentiality up front and let Maria set the pace and depth of what she shares today.

near-miss **a.** Reassure Maria that sessions are private and move directly into structured symptom questions today.

novice **c.** Encourage Maria to report the robbery to police now so the case can advance while details are fresh.

harmful **d.** Tell Maria that trust issues must be resolved before counseling can continue and defer further sessions.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

Maria's eyes fill with tears mid-sentence while describing her nightmares, and her breathing quickens. What should the counselor do next in this moment?

was: Maria becomes tearful describing nightmares. What is the best in-session response?

KEY **c.** Validate the fear, guide a brief grounding exercise, then ask if she wants to continue.

near-miss **a.** Redirect Maria to a neutral topic for the rest of the hour to keep her affect from escalating.

novice **b.** Ask Maria for a detailed account of each nightmare to build an exposure hierarchy this session.

harmful **d.** Dismiss the tears as normal adjustment, end the session early, and tell Maria to return when calmer.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

At intake Maria confirms her symptoms began within days of the robbery two weeks ago. Which finding most strongly supports acute stress disorder rather than PTSD?

was: What feature most supports acute stress disorder rather than PTSD?

KEY **d.** Onset within days of the robbery and a two-week duration, inside the one-month acute window.

near-miss **a.** Presence of intrusive memories and nightmares, which are core reexperiencing features of both disorders.

novice **b.** Report of dissociative detachment, a feature that by itself distinguishes acute stress from PTSD.

harmful **c.** Duration beyond one month, which places the reaction in the acute rather than chronic range.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R2

Maria asks what counseling will involve now that she is three weeks past the robbery. Which initial treatment plan is most appropriate?

was: What is the most appropriate early treatment for Maria?

KEY **a.** Begin trauma-focused CBT with psychoeducation, stabilization skills, and culturally responsive engagement.

near-miss **b.** Provide weekly supportive counseling and sleep hygiene coaching, deferring trauma-focused work indefinitely.

novice **c.** Schedule a single-session psychological debriefing to process the robbery before symptoms consolidate.

harmful **d.** Delay all intervention until one month has passed and PTSD criteria can be formally confirmed.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3

In Session 2 Maria leans back and asks whether anything she says could reach immigration authorities. How should the counselor respond ethically?

was: Maria fears information could reach immigration authorities. What should the clinician do?

KEY **c.** Review confidentiality protections and their specific legal limits so Maria can consent knowingly.

near-miss **a.** Assure Maria that immigration status is outside the scope of counseling and continue the interview.

novice **b.** Promise Maria that nothing she says will leave the room, since disclosure would damage the alliance.

harmful **d.** Document Maria's immigration concerns in the shared agency record and notify the referring officer.

☐ approve _____ re-tier _____ note

ncmhce-G059 Limited Treatment Access in a Rural Setting

Opioid Use Disorder, Severe · hard

Q2 · COUNSELING · R2, R3

In session 1 Daniel's voice trails off as he says the nearest methadone clinic is two hours away and he has no reliable ride. What is the most effective counseling response at this moment?

was: Daniel feels hopeless about access. What is the best counseling response?

KEY **b.** Affirm his goal to stop using and collaboratively map out which reachable options feel workable to him.

near-miss **a.** Provide a printed list of regional treatment programs and their intake numbers for him to call this week.

novice **c.** Challenge his hopeless thinking as a cognitive distortion and assign a thought record before the next visit.

harmful **d.** Advise him that treatment cannot start until he secures transport, and place his case on hold for now.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

In session 2 Daniel says he should be able to quit on willpower alone and that taking a medication would mean he is weak. How should the counselor respond?

was: Daniel says he should be able to quit on willpower alone. Best response?

KEY **c.** Explore what willpower means to him, then share that medication with counseling is proven effective.

near-miss **a.** Support a brief self-directed taper first and revisit medication if he returns still using next month.

novice **d.** Direct him to admit powerlessness at local twelve-step meetings as the required first step in recovery.

harmful **b.** Agree that medication just swaps one drug for another and encourage him to detox on his own at home.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Daniel's intake and session 1 report, which finding best supports specifying his opioid use disorder as severe?

was: Which finding most supports a severe specifier for opioid use disorder?

KEY **d.** Endorsement of at least eight criteria including tolerance, withdrawal, craving, and harm from use.

near-miss **b.** Presence of tolerance and withdrawal together, showing physiological dependence on the opioid supply.

novice **a.** Daily opioid use at escalating doses, since the amount consumed sets the severity specifier level.

harmful **c.** Job loss and family strain alone, which confirm severity regardless of how many criteria are met.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

Daniel wants to start treatment but cannot reach the methadone clinic two hours away and has limited income. Which treatment plan best fits his situation?

was: Given rural barriers, what treatment plan best fits Daniel?

KEY **a.** Start office-based buprenorphine by telehealth with counseling, plus transport and local supports.

near-miss **c.** Refer him to the methadone program two hours away and arrange vouchers for the daily dosing visits.

novice **b.** Provide weekly outpatient counseling and relapse prevention only, deferring the medication question for later.

harmful **d.** Recommend an unsupervised home detox this week, with a follow-up call once withdrawal has resolved.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R2, R3

Daniel says with audible apprehension that in his county everybody knows everybody and he fears being judged if word spreads that he is in treatment. What should the counselor do?

was: Daniel worries the small community will learn he is in treatment. What applies?

KEY **c.** Explain the heightened confidentiality rules for substance use records and how his data is protected.

near-miss **a.** Reassure him that all sessions stay confidential and revisit the topic if a release is later requested.

novice **b.** Tell him other local providers may access his records freely for care coordination under privacy law.

harmful **d.** Contact his relatives to enlist their support in his recovery, since family involvement improves outcomes.

☐ approve _____ re-tier _____ note

ncmhce-G060 Entitlement and Transition Struggles in a Veteran

Narcissistic Personality Disorder · medium

Q2 · COUNSELING · R2, R3

In session one, Marcus leans forward and states that the clinician's degree means nothing next to real combat leadership. Which response is best at this moment?

was: Marcus devalues the clinician's credentials early on. Best response?

KEY **b.** Remain nondefensive, monitor his own reactions, and invite Marcus to say more about his doubts.

near-miss **a.** Interpret the devaluing as a defense against underlying shame to build insight in this first session.

novice **c.** Assert his training credentials calmly, then redirect Marcus back to his stated job-conflict goals.

harmful **d.** Confront the disrespect firmly and warn Marcus that continued insults will end the counseling relationship.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R1

As Marcus's volume rises while calling his coworkers clueless amateurs, how should the clinician proceed in this session?

was: Marcus expresses contempt for coworkers. How should the clinician proceed?

KEY **c.** Reflect his frustration, then explore how these patterns affect his work goals.

near-miss **a.** Validate his leadership abilities and postpone exploration until the alliance feels firmer.

novice **b.** Assign assertiveness-communication homework so Marcus can address coworkers more effectively.

harmful **d.** Agree that his coworkers are incompetent and help him document their failures for management.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Marcus's intake and two sessions, which finding best supports narcissistic personality disorder over an adjustment reaction to civilian transition?

was: Which finding best distinguishes NPD from an adjustment reaction to civilian transition?

KEY **a.** Grandiosity and entitlement have been pervasive across settings since well before he enlisted.

near-miss **c.** His anger and devaluing intensified after discharge, when civilian ranking frustrated him.

novice **b.** His symptoms began within three months of a clear stressor and impair his current work role.

harmful **d.** His combat exposure and hyperarousal explain the irritability, ruling out a personality disorder.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R1

After session two confirms a longstanding pattern predating service, which treatment focus best fits Marcus?

was: What treatment focus best fits Marcus?

KEY **d.** Build a steady alliance that addresses vulnerability beneath grandiosity and grows empathy.

near-miss **b.** Focus on brief skills coaching for workplace conflict and defer character-level work.

novice **a.** Center treatment on exposure and grounding work for presumed service-related trauma symptoms.

harmful **c.** Confront his grandiosity firmly in each session to dismantle his defenses more quickly.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R2

After Marcus's smirking dismissal of civilians in session two, the clinician notices rising irritation and dislike. What is the ethical course?

was: The clinician feels irritation toward Marcus. What is the ethical course?

KEY **b.** Uphold Marcus's dignity, seek supervision, and manage his own reactive feelings.

near-miss **d.** Disclose his irritation to Marcus early to model honesty about their working relationship

novice **a.** Set firm limits on Marcus's boasting and document each violation of session ground rules.

harmful **c.** Refer Marcus out at once, noting that untreatable clients waste limited clinical resources.

☐ approve _____ re-tier _____ note

ncmhce-G061 Mood Swings and Somatic Complaints Amid Family Expectations

Bipolar II Disorder · medium

Q2 · COUNSELING · R3, R1

In Session 1, Linh says 'mostly my body just hurts and I can't sleep' and describes her mood shifts almost entirely as headaches and fatigue. Which counselor response is most appropriate?

was: Linh presents distress as headaches and fatigue. Best counseling response?

KEY **b.** Acknowledge the headaches and fatigue as real, then trace how they track with her sleep and energy shifts.

near-miss **a.** Acknowledge the physical symptoms, then defer mood tracking until a medical workup rules out causes.

novice **c.** Redirect her from body complaints and teach relaxation breathing to reduce the headaches and tension.

harmful **d.** Tell her the somatic complaints are avoidance of real feelings and press her to name the underlying emotions directly.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3

In Session 2, Linh's gaze drops as she describes pressure to meet her family's expectations and worries they would view counseling as failure. What should the counselor do next?

was: Linh fears family will see therapy as failure. Best response?

KEY **c.** Explore what her family's expectations mean to her and decide together how much to involve them.

near-miss **a.** Reassure her that therapy is confidential and revisit the family question later in the course of treatment.

novice **b.** Reframe her family's expectations as a cognitive distortion and dispute the beliefs with thought records.

harmful **d.** Advise her to limit contact with her family until her mood episodes stabilize over the next few months.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Linh's history of several-day elevated periods that coworkers noticed, which finding most clearly supports bipolar II rather than bipolar I disorder?

was: Which finding best distinguishes bipolar II from bipolar I disorder?

KEY **a.** Elevated periods lasting several days that others notice but that do not cause marked impairment.

near-miss **c.** Depressive episodes that outlast and outnumber the elevated periods across her illness course.

novice **b.** Absence of psychiatric hospitalization or medication trials during any of her elevated periods.

harmful **d.** Presentation of her mood shifts primarily as headaches and fatigue rather than emotional distress.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2

Linh's hypomanic periods consistently begin after nights of reduced sleep and family gatherings. Which treatment approach best supports her mood stability?

was: Which psychosocial intervention best supports Linh's stability?

KEY **d.** Begin interpersonal and social rhythm therapy to regularize sleep, daily routines, and role stressors.

near-miss **a.** Begin cognitive behavioral therapy for depression with weekly behavioral activation assignments.

novice **b.** Begin exposure and response prevention targeting her somatic preoccupation and health-related worry.

harmful **c.** Begin unstructured supportive counseling and encourage her to use hypomanic periods for extra productivity.

☐ approve _____ re-tier _____ note

Q6 · CORE · R1

After Session 2, the counselor writes the progress note describing Linh's hypomanic periods. What is most essential to include?

was: What is essential when documenting Linh's hypomanic episodes?

KEY **c.** Record episode duration, observable behavior change, functioning, and absence of psychotic features.

near-miss **a.** Record her self-reported mood ratings and symptom severity scores at each session in the chart.

novice **d.** Record the headache and fatigue complaints as the primary presenting problem for each episode.

harmful **b.** Record the elevated periods as manic episodes to secure insurance authorization for her ongoing treatment.

☐ approve _____ re-tier _____ note

ncmhce-G062 Mood Episodes with Psychosis and Community Disconnection

Schizoaffective Disorder, Bipolar Type · hard

Q2 · COUNSELING · R2, R3

In Session 2, Joseph's voice warms as he describes the ceremonies and community ties he has been cut off from. How should the counselor respond to this disclosure?

was: Joseph values traditional healing. How should the clinician respond?

KEY **b.** Affirm the value of ceremony and plan with Joseph how community healing can run alongside his clinical treatment.

near-miss **c.** Note his interest in ceremony in the chart and revisit cultural supports once his psychotic symptoms fully stabilize.

novice **a.** Reframe ceremony as a relaxation technique and fold it into a standard coping-skills module for psychosis.

harmful **d.** Caution Joseph that ceremony may reinforce his delusions and advise that he postpone it until symptoms remit.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

In Session 1, Joseph's tone deepens with feeling as he ties his distress to generations of loss in his community. What is the counselor's best response?

was: Joseph mentions historical trauma affecting his community. Best response?

KEY **c.** Acknowledge the impact of historical trauma and explore with Joseph how it shapes his symptoms and supports.

near-miss **d.** Validate his account briefly and redirect the session to mapping the timeline of his mood and psychotic episodes.

novice **b.** Interpret his references to history as avoidance of personal material and steer him back to individual symptoms.

harmful **a.** Tell Joseph that dwelling on past injustice fuels his paranoia and ask him to drop the topic in sessions.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

At intake Joseph states, 'the voices don't wait for my moods.' Which finding best supports schizoaffective disorder rather than bipolar I with psychotic features?

was: Which feature best distinguishes schizoaffective disorder from bipolar with psychotic features?

KEY **a.** Hallucinations and delusions persisting two or more weeks during periods when mood symptoms are absent.

near-miss **b.** Mood episodes meeting full criteria for mania and major depression across the course of his illness.

novice **c.** Psychotic content that is mood-congruent and intensifies as his manic energy and grandiosity increase.

harmful **d.** Duration of psychosis under two weeks between his mood episodes, which meets the schizoaffective threshold.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R3, R2

Joseph's mood has been stable for six weeks and he asks what his ongoing care should include. Which treatment plan best fits his presentation?

was: What treatment approach best fits Joseph?

KEY **d.** Pair antipsychotic and mood-stabilizing medication with psychosocial rehabilitation and cultural supports.

near-miss **a.** Continue medication management alone and defer psychosocial and cultural referrals until he has a year of stability.

novice **b.** Provide weekly insight-oriented psychotherapy focused on mood episodes without antipsychotic medication.

harmful **c.** Discontinue antipsychotic coverage between mood episodes and rely on ceremony attendance to manage the voices.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R2, R3

Joseph asks that a traditional healer from his home community take part in his treatment planning. What is the counselor's ethically appropriate action?

was: Joseph requests including a traditional healer in his care. What is appropriate?

KEY **c.** Obtain written consent and coordinate respectfully with the healer as a partner in Joseph's care plan.

near-miss **a.** Encourage Joseph to continue seeing the healer privately while keeping the two treatments separate.

novice **d.** Ask the healer to document sessions and follow the clinic's treatment protocol before any contact occurs.

harmful **b.** Share Joseph's chart with the healer without consent so the community can help monitor his symptoms.

☐ approve _____ re-tier _____ note

ncmhce-G063 Progressive Memory Decline and Caregiver Strain

Major Neurocognitive Disorder due to Alzheimer's Disease · medium

Q2 · COUNSELING · R2

During the first session, Dorothy's son describes the daily care demands in a weary, strained voice, his eyes tired. Which counselor response is most appropriate at this moment?

was: The son appears exhausted and overwhelmed. Best counseling response?

KEY **c.** Acknowledge the strain he carries, map his current supports, and explore accessible respite options.

near-miss **a.** Refer him promptly to a caregiver support group before reviewing the supports he already relies on.

novice **b.** Redirect the session to Dorothy's cognitive symptoms and address his fatigue at a later visit.

harmful **d.** Instruct him to place Dorothy in a nursing facility now, since his own disability makes home care unsafe.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

In the second session, Dorothy's son's voice catches as he says he feels guilty asking for help because of his own disability. What is the counselor's best response?

was: The son feels guilty about needing help due to his own disability. Best response?

KEY **a.** Normalize help-seeking, affirm the care he provides, and reframe outside support as sustainable.

near-miss **d.** Explore the roots of his guilt in depth before discussing any concrete support arrangements.

novice **b.** Challenge his guilt as an irrational belief and assign a thought record to dispute it weekly.

harmful **c.** Agree that his own disability limits him and recommend a relative assume Dorothy's daily care now.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Dorothy's two-year history and her presentation across both sessions, which finding most strongly argues against delirium?

was: Which feature best distinguishes this disorder from delirium?

KEY **d.** Her decline has advanced steadily across two years with clear, stable attention at each visit.

near-miss **b.** Her memory loss is severe enough to interfere with independent daily functioning at home.

novice **a.** Her flat affect and slow, vague speech rule out any depressive contribution to decline.

harmful **c.** Her age above 75 confirms a degenerative cause and excludes any reversible medical contributors.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R2

Given Dorothy's progressive, irreversible decline and her son's reported burden, what should the initial treatment plan emphasize?

was: What is the most appropriate care plan focus?

KEY **b.** Build a plan for supportive routines, home safety, caregiver support, and accessible services.

near-miss **d.** Center the plan on weekly cognitive stimulation groups to slow Dorothy's memory decline.

novice **a.** Center the plan on insight-oriented therapy to help Dorothy process her losses and grief.

harmful **c.** Defer safety and support planning until repeat testing confirms the stage of Dorothy's illness.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3

Dorothy's son, who uses a wheelchair, must reach the counselor's office by a flight of stairs. What is the counselor ethically required to do?

was: The son uses a wheelchair and the office has stairs. What is required?

KEY **c.** Arrange an accessible meeting space or telehealth so the son can attend sessions equitably.

near-miss **a.** Offer home visits for now and postpone any change to the office accessibility plan.

novice **b.** Ask the son to bring a helper who can assist him up the office stairs each session.

harmful **d.** Meet only with Dorothy and refer the son elsewhere, since the office lacks step-free access.

☐ approve _____ re-tier _____ note

ncmhce-G064 Persistent Physical Concerns and Faith-Based Meaning

Somatic Symptom Disorder · easy

Q2 · COUNSELING · R2, R3

In session one David's tone softens as he describes his stomach and back pain as a spiritual test given to him. How should the counselor respond in that moment?

was: David frames his pain as a spiritual test. Best counseling response?

KEY **b.** Explore with David how his spiritual meaning of the pain can support daily coping and functioning.

near-miss **a.** Redirect David to symptom-monitoring logs first and revisit the spiritual meaning in later sessions.

novice **c.** Reframe the spiritual test belief as a cognitive distortion that maintains his focus on body pain.

harmful **d.** Advise David that faith-based interpretation of pain delays healing and should be set aside in therapy.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R1, R3

David circles back repeatedly to bodily details and presses the counselor to confirm that nothing serious was missed on his scans. What is the counselor's best response?

was: David seeks repeated reassurance about his physical symptoms. Best response?

KEY **c.** Validate that his pain is real while shifting the focus toward coping skills and daily functioning.

near-miss **a.** Reflect his fear and wait for medical records before introducing coping and functioning goals.

novice **b.** Answer each of his questions about the test results in detail so his worry drops before coping work.

harmful **d.** Tell David the tests prove nothing is wrong and that further symptom talk will end the sessions.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

David presents with genuinely painful stomach and back symptoms plus excessive symptom-focused worry for over six months. Which feature best supports somatic symptom disorder rather than illness anxiety disorder?

was: Which feature best distinguishes somatic symptom disorder from illness anxiety disorder?

KEY **d.** Distressing somatic symptoms are present along with excessive symptom-focused thoughts and behavior.

near-miss **a.** Elevated health anxiety and time-consuming care seeking are present in both of these diagnoses.

novice **b.** Preoccupation with having a serious illness persists while somatic symptoms remain minimal or absent.

harmful **c.** Medical testing finds no organic cause, so the reported pain is best treated as intentionally produced.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

After David asks in session two that his faith be respected within care, which treatment plan should the counselor propose?

was: What treatment approach best fits David?

KEY **a.** Begin CBT targeting symptom-focused thoughts and reassurance behaviors, incorporating his faith values.

near-miss **c.** Begin supportive counseling that focuses on emotional expression about his pain and clergy support.

novice **b.** Begin exposure therapy for panic attacks with breathing retraining aimed at reducing feared bodily cues.

harmful **d.** Begin coordinating further specialty referrals and imaging until a physical cause of the pain is found.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R2

David leans forward in session two and firmly asks that his religious beliefs be honored throughout treatment. Which counselor action reflects ethical practice?

was: David wants his religious beliefs respected in treatment. What applies?

KEY **c.** Respect his beliefs, avoid imposing personal values, and integrate faith where clinically indicated.

near-miss **a.** Acknowledge his beliefs briefly, then keep the sessions limited to symptom management and coping skills.

novice **b.** Transfer David to a pastoral counselor since religious concerns fall outside professional counseling.

harmful **d.** Disclose the counselor's own doctrine and pray with David to align treatment with religious teaching.

☐ approve _____ re-tier _____ note

ncmhce-G065 Rule-Breaking Behavior and Systemic Pressures

Conduct Disorder, Adolescent-Onset Type · medium

Q2 · COUNSELING · R2, R3

In the first session Andre sits with arms crossed, answers in clipped phrases, and mutters that school staff 'always come at me first.' What is the counselor's best response at this point?

was: Andre is guarded and mistrustful of the clinician. Best response?

KEY **c.** Name that his wariness makes sense given his history, and let trust build at his pace with steady respect.

near-miss **a.** Move directly into a behavior contract for fighting and truancy before the working alliance is established.

novice **b.** Confront his guardedness as resistance and press him to admit responsibility for the fights right away.

harmful **d.** Warn him that continued guardedness will be reported to the court and school as refusal to cooperate.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

In session two Andre describes racial bias in how his school assigns suspensions and says 'nobody ever asks why.' How should the counselor respond?

was: Andre describes racial bias in school discipline. Best response?

KEY **b.** Affirm that the unfair treatment he names is real, then explore together what choices remain open to him.

near-miss **a.** Focus the session on documenting each discipline incident so a formal complaint can be filed with the district.

novice **c.** Redirect him toward cognitive restructuring of his belief that staff single him out for harsher punishment.

harmful **d.** Tell him that blaming racism keeps him stuck and that owning his behavior is the only real path forward.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Andre's records show fighting, truancy, and rule violations over the past year. Which feature most clearly supports conduct disorder rather than oppositional defiant disorder?

was: Which feature best distinguishes conduct disorder from oppositional defiant disorder?

KEY **d.** A repeated pattern of aggression, deceit, and rule violation breaching others' rights and social norms.

near-miss **a.** A one-year duration of symptoms with impairment in school functioning and repeated disciplinary referrals.

novice **b.** A pattern of angry mood, argumentativeness, and defiance toward adults in positions of visible authority.

harmful **c.** A symptom onset before age ten that rules out oppositional defiant disorder as a plausible diagnosis here.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R2

With conduct disorder, adolescent-onset type established and Andre living with family while enrolled in school, which treatment plan is most appropriate?

was: What treatment approach best fits Andre?

KEY **a.** Engage family, school, and community systems together in an evidence-based intervention that builds skills.

near-miss **c.** Begin weekly individual anger management sessions with Andre and defer any family or school involvement.

novice **b.** Refer Andre to a peer group program for delinquent youth as the single core component of his treatment.

harmful **d.** Recommend residential placement now and advise the school to pursue expulsion to enforce clear consequences.

☐ approve _____ re-tier _____ note

Q6 · CORE · R2, R3

Before finalizing Andre's diagnosis, what is most essential for the counselor to do to guard against diagnostic bias?

was: What is essential to avoid bias when diagnosing Andre?

KEY **b.** Assess how harsh discipline and racial bias shaped his behavior before attributing it to internal pathology.

near-miss **d.** Rely on standardized behavior rating scales from teachers and parents to confirm the diagnostic criteria.

novice **a.** Count each documented rule violation in his school record and apply the criteria without any contextual review.

harmful **c.** Assign the childhood-onset subtype since youth from high-crime neighborhoods show earlier behavioral onset.

☐ approve _____ re-tier _____ note

ncmhce-G066 Self-Injury Amid Family Shame and Language Barriers

Nonsuicidal Self-Injury · hard

Q2 · COUNSELING · R1, R2

In her first session Sofia whispers that cutting 'makes the feelings stop for a minute' while glancing toward the door. Which counselor response is best at this moment?

was: Sofia discloses self-injury and seems anxious. Best initial response?

KEY **c.** Respond calmly, validate the distress behind the cutting, and explore what feelings it relieves for her.

near-miss **a.** Move quickly to teaching replacement behaviors so she has alternatives to cutting before the session ends.

novice **d.** Express alarm at the injuries and urge her to promise she will stop cutting before the next session.

harmful **b.** Warn her that continued cutting means hospitalization and phone her parents about it right now.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R1, R4

Sofia lowers her voice and says she is afraid of bringing shame to her family if they learn she has been cutting. How should the counselor respond?

was: Sofia fears her family will be ashamed. Best response?

KEY **a.** Validate the fear, explore what self-injury means in her family, and plan any disclosure together.

near-miss **b.** Focus on building her coping skills first and revisit the family shame concern later in treatment.

novice **d.** Reassure her that her parents will understand once the behavior is explained as a symptom of anxiety.

harmful **c.** Tell her that cultural shame beliefs are unhelpful and encourage her to disclose to her parents tonight.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R2, R1

At intake Sofia reports that cutting relieves overwhelming feelings and states she does not want to die. What is the assessment priority?

was: What is the priority during Sofia's intake assessment?

KEY **d.** Screen systematically for suicidal ideation, plans, and prior attempts using a standardized measure.

near-miss **a.** Document the frequency, methods, and triggers of her self-injury episodes over the past twelve months.

novice **b.** Explore the family dynamics and acculturation stressors that appear to be driving her emotional distress.

harmful **c.** Accept her denial of suicidal intent and move directly to skills training in the remaining intake time.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R3, R1

In session two Sofia explains in Spanish that her cutting relieves distress rather than being an attempt to end her life. What is the most appropriate next step in her care?

was: What is the most appropriate next step in Sofia's care?

KEY **b.** Build a collaborative safety plan in Spanish and teach distress-tolerance skills she can use at home.

near-miss **a.** Begin weekly individual therapy focused on insight into the origins of her overwhelming emotions.

novice **c.** Refer her for medication evaluation to reduce the urges before any coping skills work is introduced.

harmful **d.** Arrange inpatient hospitalization now and pause outpatient work until her self-injury has fully stopped.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R4

Sofia asks in Spanish whether the things she tells the counselor will be reported to her parents. Which action best meets the counselor's ethical obligations?

was: Sofia is a Spanish-dominant minor worried about confidentiality. What applies?

KEY **c.** Explain confidentiality and its limits in Spanish, using a qualified interpreter when needed.

near-miss **a.** Explain the limits of confidentiality later, once she has settled and trusts the counseling relationship.

novice **d.** Explain confidentiality in English and ask her bilingual older cousin to interpret the key points.

harmful **b.** Promise her that nothing she reveals will be shared with her parents under any circumstance.

☐ approve _____ re-tier _____ note

ncmhce-G067 New Parent Facing Postpartum Distress

Major Depressive Disorder, with Peripartum Onset · medium

Q2 · COUNSELING · R1, R3

At intake Amanda says quietly, "I keep waiting to feel like her mother," while describing guilt about not bonding. What is the counselor's best initial response?

was: What is the most appropriate initial counseling response to her disclosure of distress?

KEY **b.** Reflect her bonding grief, normalize peripartum mood symptoms, and explore the minority-stress pressures she names.

near-miss **a.** Reassure her that bonding feelings arrive naturally for most new parents and revisit the concern at the next appointment.

novice **c.** Begin structured sleep-hygiene coaching to correct her disrupted rest before addressing the mood content.

harmful **d.** Advise her that her wife's bond with the infant is primary and suggest stepping back from nighttime caregiving.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3

In session 1 Amanda's voice breaks as she describes feeling invisible as the non-gestational parent while relatives question her role. How should the counselor address this?

was: How should the counselor best address her sense of invisibility as the non-gestational parent?

KEY **c.** Explore how family invalidation shapes her parenting identity and affirm her legitimacy as the child's parent.

near-miss **b.** Encourage her to schedule more solo caregiving time so bonding builds before processing family reactions.

novice **a.** Challenge her belief that she is invisible as a cognitive distortion and assign a thought-record log.

harmful **d.** Suggest she accept a secondary caregiver role for now, framing her invisibility as a temporary depressive distortion.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Amanda reports that low mood, anhedonia, guilt, and fatigue have occurred daily since her child's birth six weeks ago. Which intake step best separates peripartum MDD from an adjustment disorder?

was: Which assessment step most clearly distinguishes peripartum MDD from an adjustment disorder?

KEY **d.** Document the number of depressive symptoms, their daily two-week duration, and the degree of functional impairment.

near-miss **a.** Document the timing of the birth to establish that onset occurred within the peripartum specifier window.

novice **b.** Document her subjective distress level relative to the stress of new parenthood using a brief rating scale.

harmful **c.** Document that childbirth is an identifiable stressor and assign an adjustment disorder diagnosis without further screening.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R2

Amanda's symptoms meet episode criteria and she endorses passive thoughts without plan or intent. Which treatment plan best fits her presentation?

was: What is the most appropriate evidence-based treatment plan for this client?

KEY **a.** Provide peripartum psychoeducation, begin structured CBT or IPT, monitor risk, and refer for medication evaluation.

near-miss **b.** Provide peripartum psychoeducation and supportive counseling, deferring structured therapy until sleep improves.

novice **c.** Provide relaxation training and parenting skills classes as the primary intervention, with monthly check-ins.

harmful **d.** Provide brief supportive sessions and postpone risk monitoring and medical referral until symptoms clearly worsen over time.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R2, R3

In session 2 Amanda says she fears being judged an unfit parent if she discloses her struggles. What is the ethically sound counselor response?

was: She fears disclosure could lead to her being judged an unfit parent. What is the ethical response?

KEY **c.** Review the limits of confidentiality, normalize help-seeking, and continue collaborative safety assessment.

near-miss **b.** Affirm that seeking help signals responsible parenting and defer the confidentiality review to a later session.

novice **a.** Promise her that nothing she reports will leave the room so she can speak about her thoughts freely.

harmful **d.** Report her passive thoughts to child protective services to document that the infant may be at risk.

☐ approve _____ re-tier _____ note

ncmhce-G068 Immigrant Client With Intense Specific Fear

Specific Phobia · easy

Q2 · COUNSELING · R2, R1

At intake Bao says quietly, 'in my village we learned dogs bring danger,' and mentions reluctance to seek care; how should the counselor open discussion of his fear?

was: What is the most culturally responsive way to open the conversation about his fear?

KEY **c.** Invite Bao to describe what dogs meant in his village while affirming his decision to come for help

near-miss **a.** Invite Bao to rate his fear on a 0-to-10 scale so severity can be tracked before exploring meaning

novice **b.** Invite Bao to review the irrational thoughts behind his fear so cognitive distortions can be corrected

harmful **d.** Invite Bao to accept that village teachings about dogs are superstitions he must set aside to improve

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

In session 1 Bao lowers his voice and says that coming to counseling marks him as weak among his community; what is the counselor's best response?

was: He says seeking counseling makes him feel weak. How should the counselor respond?

KEY **a.** Frame reaching out as an act of courage and explore how community expectations shape his hesitation

near-miss **b.** Frame the stigma as common and move directly into building the exposure hierarchy for his dog fear

novice **c.** Frame his belief as a distorted thought and dispute it with evidence that counseling clients improve

harmful **d.** Frame the concern as resistance and warn Bao that continued doubt will require closing his case

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

In session 2 Bao details when and how his anxiety arises; which finding most supports specific phobia over panic disorder?

was: Which finding best supports specific phobia over panic disorder?

KEY **d.** His surges of anxiety occur only when a dog is present or expected, not out of the blue

near-miss **a.** His avoidance of parks and streets has narrowed his routine and left him more isolated

novice **b.** His panic includes sweating and quickened breathing with a fear that danger is imminent

harmful **c.** His fear has lasted years and he calls it excessive, which rules out any panic diagnosis

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R3, R2

Bao agrees to work on his dog fear so he can walk to work again; which treatment approach should the counselor propose first?

was: What is the recommended first-line treatment approach for his presentation?

KEY **b.** Begin graded in vivo exposure to dogs, pacing each step with Bao and adding culturally framed teaching

near-miss **a.** Begin relaxation and breathing training alone, deferring any contact with dogs until anxiety subsides

novice **c.** Begin weekly insight-oriented sessions exploring childhood village memories to resolve the fear's roots

harmful **d.** Begin an unannounced session with a large dog in the room so Bao learns his fear is groundless

☐ approve _____ re-tier _____ note

Q6 · CORE · R1

Reviewing Bao's report across both sessions, which statement best captures the core features of his diagnosis?

was: Which statement best reflects the core clinical features of his diagnosis?

KEY **c.** A marked cued fear of dogs, judged excessive, driving avoidance and impairment for over six months

near-miss **a.** A marked fear of dogs with immediate panic responses that has continued for more than six months

novice **b.** A pervasive worry across many situations with physical tension that has lasted more than six months

harmful **d.** A culture-bound belief about dogs that is normative in his village and not a treatable disorder

☐ approve _____ re-tier _____ note

ncmhce-G069 Rural Widow Struggling After Loss

Prolonged Grief Disorder · medium

Q2 · COUNSELING · R1, R2

During the first session Wilma murmurs, 'I still set two cups out in the morning,' then trails into silence. Which counselor response is most therapeutic at this moment?

was: What is the most therapeutic initial response to her ongoing yearning?

KEY **b.** Reflect the meaning of the two cups and invite her to describe how the loss changed her daily routines

near-miss **a.** Normalize her yearning as a common stage of bereavement and move on to reviewing her sleep and appetite

novice **d.** Assign daily thought records to dispute her belief that life without her husband is now meaningless

harmful **c.** Direct her to pack away his belongings and stop setting the second cup to break the attachment

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3, R2

Wilma sighs that geographic isolation limits her contact with others and that she drives over an hour to reach any provider. How should the counselor address this in the ongoing work?

was: How should the counselor address her geographic isolation in the work?

KEY **c.** Name the distance barriers and jointly weigh telehealth visits and nearby supports she can reach weekly

near-miss **a.** Focus sessions on grief content and revisit her transportation barriers if her attendance becomes irregular

novice **d.** Reduce her to monthly in-person visits so the long drive feels manageable, without added contact

harmful **b.** Advise her to relocate nearer to the clinic and defer grief work until she lives in town

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1, R2

In the second session Wilma steadies her voice and denies pervasive mood disturbance unrelated to the loss. Which finding most clearly supports prolonged grief disorder rather than major depressive disorder?

was: Which feature most clearly supports prolonged grief over major depression?

KEY **a.** Her distress is organized around yearning and preoccupation with her husband rather than pervasive low mood

near-miss **c.** Her grief symptoms have lasted eighteen months, exceeding the twelve-month duration threshold for the diagnosis

novice **d.** Her tearfulness, numbness, and low energy show a mood episode with prominent bereavement features

harmful **b.** Her avoidance of reminders and numbness confirm posttraumatic stress from witnessing his death

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

With prolonged grief disorder established at eighteen months post-loss, which treatment plan best fits Wilma's presentation and her remote location?

was: What treatment approach is most appropriate for prolonged grief disorder?

KEY **d.** Deliver grief-focused therapy targeting yearning, avoidance, and meaning, adapted for telehealth delivery

near-miss **b.** Begin general supportive counseling for bereavement and add grief-specific interventions in several months

novice **a.** Provide prolonged exposure to trauma memories of his illness and death as the core treatment focus

harmful **c.** Refer her for inpatient stabilization and suspend outpatient grief work until symptoms fully remit

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R1

Wilma's county has no grief specialist within an hour's drive, and she is unlikely to travel farther. What is the ethically sound course of action for her counselor?

was: Given limited rural services, what is the ethically sound course of action?

KEY **b.** Practice within her competence in grief care while using consultation and telehealth to close access gaps

near-miss **d.** Continue weekly sessions and delay seeking any consultation until she shows limited response to grief treatment

novice **a.** Refer her out to a distant grief specialist since rural travel makes consistent counseling impractical

harmful **c.** Provide telehealth grief work without verifying licensure rules or arranging local emergency backup

☐ approve _____ re-tier _____ note

ncmhce-G070 Veteran Using Stimulants Under Stress

Stimulant Use Disorder (Cocaine) · medium

Q2 · COUNSELING · R2, R3

In session two Eric says his cocaine use is damaging his health and marriage yet states he is not ready to commit to quitting; which counseling stance should the counselor adopt at this moment?

was: Given his ambivalence, what is the most appropriate counseling stance?

KEY **b.** Reflect both sides of Eric's ambivalence and invite him to voice his own reasons for changing his use.

near-miss **a.** Provide Eric clear feedback about cocaine's health effects and ask permission before adding more advice.

novice **c.** Confront Eric's minimizing directly so he accepts the label of addiction before treatment proceeds.

harmful **d.** Warn Eric that continued use will end counseling and require discharge to a locked detox unit.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2

Eric shrugs and says cocaine 'just takes the edge off so I can keep performing'; which counselor response is most likely to elicit change talk from him?

was: Which response best evokes change talk from this client?

KEY **c.** Ask Eric what worries him most about his use and how he would want his marriage to look.

near-miss **a.** Summarize the pros and cons Eric listed and reflect back the strain he named at home.

novice **b.** Ask Eric how many grams he uses weekly and whether he now agrees that he is addicted.

harmful **d.** Tell Eric his spouse will leave unless he quits cocaine before the next scheduled session.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Eric's intake and first session together, which cluster of findings most directly establishes the stimulant use disorder diagnosis?

was: Which assessment finding most directly supports a stimulant use disorder?

KEY **a.** Craving, larger amounts needed for effect, and use continuing despite conflict with his wife.

near-miss **b.** Considerable time spent obtaining the drug, with job duties still completed at his usual level.

novice **c.** Restless leg movement and steady eye contact at intake, consistent with autonomic hyperarousal.

harmful **d.** His composed presentation and steady employment, which rule out a substance use disorder diagnosis.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2, R3

Given that Eric prefers community-based care and is not yet willing to commit to abstinence, what should the initial treatment plan emphasize?

was: Considering his readiness, what is the best initial treatment focus?

KEY **d.** Focus early sessions on motivational work and a decisional balance matched to his contemplation.

near-miss **a.** Begin contingency management with drug screens to reinforce the abstinence he has not yet chosen.

novice **b.** Begin stress management and trauma processing first, treating cocaine use as a coping symptom.

harmful **c.** Require a signed abstinence contract and mandatory VA inpatient care as a condition of treatment.

☐ approve _____ re-tier _____ note

Q6 · CORE · R3

In session two Eric weighs the pros and cons of quitting aloud while declining to commit to abstinence; which stage-of-change formulation best fits him?

was: Which statement best reflects his current stage of change?

KEY **b.** Contemplation: he recognizes harm to health and marriage yet withholds commitment to action.

near-miss **d.** Preparation: he is weighing options and appears close to setting a change date within weeks.

novice **a.** Precontemplation: his minimizing and shrugging show he sees no problem with his cocaine use.

harmful **c.** Action: his steady job and controlled presentation show he has already changed his use pattern.

☐ approve _____ re-tier _____ note

ncmhce-G071 Young Man With Persistent Rule-Breaking

Antisocial Personality Disorder · hard

Q2 · COUNSELING · R3, R1

In session 1 Kevin smirks while describing how he has exploited others and calls rules 'my family's way of controlling me.' Which counseling stance should the counselor take with him?

was: What is the most effective counseling stance with this client?

KEY **c.** Hold clear, consistent limits with Kevin while staying warmly engaged and nonjudgmental about his account.

near-miss **a.** Prioritize building rapport by deferring limit-setting until Kevin reports trusting the counselor.

novice **b.** Confront Kevin's smirking and lack of remorse directly to break down his denial in each session.

harmful **d.** Relax session limits and accommodate Kevin's demands so that he does not disengage from the counseling process.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R1

Kevin explains that his rule-breaking is rebellion against rigid collectivist family expectations. How should the counselor respond to this framing?

was: How should the counselor handle his framing of behavior as cultural rebellion?

KEY **b.** Explore the family-pressure context while assessing whether his pattern spans multiple settings.

near-miss **a.** Focus the session on acculturative stress and rigid family expectations before reviewing his behavior.

novice **c.** Correct his cultural explanation and reframe each rule violation as a symptom of his personality disorder.

harmful **d.** Accept his account of cultural rebellion and defer the personality assessment to avoid cultural bias.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Kevin is 29 and reports deceit, impulsivity, irritability, and disregard for others since his teens. Which finding must the counselor confirm before assigning antisocial personality disorder?

was: Which finding is required to diagnose antisocial personality disorder?

KEY **d.** Documented evidence of conduct disorder with onset before Kevin's fifteenth birthday.

near-miss **a.** Documented deceitfulness, impulsivity, and reckless disregard for the safety of others.

novice **b.** Documented arrest history with criminal charges filed before Kevin reached adulthood.

harmful **c.** Documented psychotic or manic episodes that could account for Kevin's rule-breaking conduct.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

Kevin externalizes blame and shows little concern for the harm he has caused. Which treatment goal should the counselor set for the initial phase of work?

was: What is the most realistic treatment goal for this client?

KEY **a.** Set specific, structured behavioral targets with harm-reduction aims Kevin can be held to weekly.

near-miss **b.** Set insight-oriented goals aimed at building Kevin's empathy and remorse before addressing behavior.

novice **c.** Set goals centered on processing childhood family conflict to resolve the roots of his rule-breaking.

harmful **d.** Set a goal of full personality change and discharge Kevin if he fails to meet it within three months.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R1

During a later session Kevin describes, with a dismissive smirk, a specific plan to hurt a former business partner he names. What is the counselor's ethical priority?

was: He reveals plans involving potential harm to others. What is the ethical priority?

KEY **c.** Assess the threat's seriousness, act to protect identifiable others, and document each decision.

near-miss **a.** Explore the disclosure clinically in session and revisit the risk at Kevin's next scheduled visit.

novice **b.** Report the disclosure to Kevin's probation officer at once as a routine condition of his referral.

harmful **d.** Keep the disclosure confidential to preserve the alliance and continue Kevin's routine sessions.

☐ approve _____ re-tier _____ note

ncmhce-G072 Elder Whose Home Is Filling Up

Hoarding Disorder · medium

Q2 · COUNSELING · R2, R1

In session 1 Margaret's tone tightens when discarding is mentioned, and she notes that some items carry ceremonial meaning while others sit unused. How should the counselor begin work on the clutter?

was: What is the most respectful way to begin addressing the clutter?

KEY **b.** Explore item meaning together, distinguishing ceremonial belongings from unused, burdensome clutter.

near-miss **a.** Begin with a room-by-room clutter inventory rating severity before discussing what any item means to her.

novice **c.** Teach exposure hierarchies for discarding distress before exploring what her possessions represent.

harmful **d.** Arrange a family cleanout day so relatives remove unused items while Margaret waits in another room.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

Margaret says quietly that she is reluctant to involve outside agencies because of what such agencies have done to her community. What is the counselor's best response?

was: How should the counselor respond to the elder's mistrust of outside agencies?

KEY **c.** Validate the historical basis of her wariness and explore culturally grounded community supports she trusts.

near-miss **a.** Reassure her the referral is voluntary and revisit outside services once rapport has been firmly established.

novice **d.** Explain that agency involvement is standard practice for hoarding and outline the referral steps for her.

harmful **b.** Contact adult protective services now so the cluttered home is evaluated without alarming Margaret first.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

In session 2 Margaret clarifies what drives her accumulation. Which finding best supports hoarding disorder rather than obsessive-compulsive disorder?

was: Which feature best distinguishes hoarding disorder from OCD here?

KEY **d.** Saving reflects a perceived need to keep items, with distress at discarding and no intrusive obsessions.

near-miss **a.** Clutter now blocks functional rooms and strains family ties, showing clear impairment from the saving.

novice **b.** Discarding provokes anxiety that is relieved by keeping items, a ritual pattern typical of compulsions.

harmful **c.** Her recall of item histories is fading, indicating accumulation driven by an emerging neurocognitive decline.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R3

Margaret agrees to work on the congested rooms but asks that nothing move quickly. Which treatment plan best fits her presentation?

was: What is the most appropriate treatment approach for this elder?

KEY **a.** Use CBT skills training for sorting, decision-making, and gradual discarding at a pace Margaret sets.

near-miss **c.** Provide supportive counseling on grief and attachment to belongings until her distress about clutter eases.

novice **b.** Deliver exposure and response prevention aimed at obsessions and compulsive checking rituals at home.

harmful **d.** Schedule a rapid professional cleanout of the home, then begin counseling on coping with the changes.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3

The relative who referred Margaret phones the counselor and asks what Margaret has said about the home during sessions. How should the counselor proceed?

was: The relative requests the counselor disclose details about sessions. What is appropriate?

KEY **b.** Decline to share content and let Margaret decide what, if anything, is released by written consent.

near-miss **d.** Offer general education about hoarding disorder to the relative while withholding Margaret's disclosures.

novice **a.** Share brief updates since the relative made the referral and is helping coordinate Margaret's care.

harmful **c.** Invite the relative to the next session and review Margaret's records together to plan a cleanup.

☐ approve _____ re-tier _____ note

ncmhce-G073 Client Navigating Body Image and Eating

Bulimia Nervosa · medium

Q2 · COUNSELING · R2, R1

In session 1, Riley says in a dropping voice, "I feel like my body is never mine to judge fairly," describing appearance judgments filtered through an ableist lens. What is the counselor's most affirming next response?

was: What is the most affirming initial response to their body-image distress?

KEY **a.** Validate the distress and explore how ableist appearance messages shape Riley's body evaluation.

near-miss **b.** Acknowledge the distress briefly and move to charting binge and purge episodes for the week.

novice **c.** Challenge the distorted body thought with cognitive disputation before exploring its context.

harmful **d.** Tell Riley that appearance concerns are secondary to disability adjustment and defer body-image work.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R1

Riley discloses, with frequent self-critical asides, that shame keeps the eating episodes entirely hidden from everyone. How should the counselor respond to this secrecy?

was: How should the counselor address the secrecy and shame surrounding eating?

KEY **c.** Normalize the shame, build safety, and explore what the binge-purge cycle does for Riley.

near-miss **a.** Praise Riley's disclosure and assign a daily food and behavior log to track the hidden episodes.

novice **b.** Interpret the secrecy as resistance and set a contract requiring full disclosure each session.

harmful **d.** Confront the concealment as dishonesty and warn that hiding episodes will end the counseling.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Riley's intake and the session 2 clarification about weight, which finding most strongly differentiates this presentation from anorexia nervosa?

was: Which feature best supports bulimia nervosa over anorexia nervosa?

KEY **d.** Weight in the typical range alongside recurrent binges and compensatory behaviors.

near-miss **a.** Self-evaluation unduly influenced by body shape and persistent distress about appearance.

novice **b.** Presence of purging behavior after eating episodes, which occurs only in bulimia nervosa.

harmful **c.** Shame-driven secrecy about eating, which rules out any restricting-type eating pathology.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R3, R1

In session 2, Riley steadies and expresses cautious readiness to work on the binge-and-compensation cycle in an affirming setting. Which treatment approach is best supported by evidence?

was: What is the recommended evidence-based treatment for this presentation?

KEY **b.** CBT for eating disorders addressing the binge-purge cycle and body image, offered accessibly.

near-miss **a.** Interpersonal psychotherapy focused on role transitions related to disability and relationships.

novice **c.** Supportive counseling for shame and self-esteem without structured symptom-focused intervention.

harmful **d.** Structured calorie-restricted meal plan with weekly weigh-ins to control the binge episodes.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R2, R3

Riley states they will engage in treatment only in an affirming, non-stigmatizing environment that accounts for their disability. Which action best meets the counselor's ethical obligations?

was: How should the counselor ensure ethical, accessible care for this client?

KEY **a.** Provide accessible formats and affirming language, setting goals collaboratively with Riley.

near-miss **b.** Refer Riley to a disability specialist for accommodations before beginning eating disorder work.

novice **c.** Apply the standard clinic protocol uniformly, assuming equal treatment ensures fair care.

harmful **d.** Decide the treatment goals for Riley and disclose the diagnosis to family without consent.

☐ approve _____ re-tier _____ note

ncmhce-G074 Devout Client Reporting Shifts in Self

Dissociative Identity Disorder · hard

Q2 · COUNSELING · R2, R3

During the first session Naomi's voice and posture shift and she whispers that she fears the shifts mean she is 'not faithful.' What is the counselor's best response at this moment?

was: How should the counselor respond to the client's faith-based interpretation?

KEY **b.** Affirm her faith framework and link it with trauma-informed language about dissociative identity states.

near-miss **a.** Focus first on psychoeducation about dissociation, deferring discussion of her religious meanings for later.

novice **c.** Reassure her that faith concerns are outside counseling and redirect her to symptom tracking homework.

harmful **d.** Interpret her identity states as a spiritual failing and recommend clergy-led correction instead of therapy.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R3, R2

In session two Naomi says she values co-consciousness language and does not want fusion as an outcome. How should the counselor proceed with goal setting?

was: The client prefers co-consciousness over integration. How should the counselor proceed?

KEY **c.** Set goals collaboratively, supporting safety and functioning while honoring her co-consciousness aim.

near-miss **a.** Set goals around symptom monitoring first, revisiting her stated identity-work preferences after stabilization.

novice **b.** Set integration as the treatment goal and educate her that fusion is the standard endpoint of care.

harmful **d.** Set aside her stated preference and contract with each identity state separately without a shared plan.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

Reviewing Naomi's intake data, which finding most clearly supports dissociative identity disorder rather than schizophrenia?

was: Which feature most clearly supports dissociative identity disorder over schizophrenia?

KEY **d.** Distinct identity states with discontinuity in agency plus recurrent amnesia for everyday events.

near-miss **a.** Reported distress and shame about the shifts, with fear of being judged as unfaithful or crazy.

novice **b.** Variable eye contact with shifts in voice and posture observed during the intake interview.

harmful **c.** Experiences of internal voices interpreted through a faith lens, indicating a primary psychotic process.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R1, R2

Considering Naomi's recurrent amnesia, guardedness, and trauma history, what should the counselor prioritize in the opening phase of treatment?

was: What is the most appropriate early treatment focus for this client?

KEY **a.** Build safety, stabilization, and trust before phased trauma processing that honors her faith values.

near-miss **b.** Begin mapping identity states and their functions now, with grounding skills added as sessions proceed.

novice **c.** Start prolonged exposure to the earliest trauma memories to reduce her dissociative symptoms quickly.

harmful **d.** Use hypnotic age regression to recover memories rapidly and confront the alternate states directly.

☐ approve _____ re-tier _____ note

Q6 · ETHICS · R3, R2

Naomi asks whether counseling will require her to give up her religious understanding of the self. Which counselor action best upholds ethical practice here?

was: Which action best upholds ethical practice with this faith-observant client?

KEY **b.** Review informed consent, respect her values, and seek consultation on complex dissociative care.

near-miss **a.** Review the treatment rationale in detail and proceed alone, documenting her faith-related concerns.

novice **c.** Refer her to a pastoral counselor at intake, since faith concerns fall outside clinical competence.

harmful **d.** Advise her that recovery requires setting aside religious beliefs, and begin identity fusion work.

☐ approve _____ re-tier _____ note

ncmhce-G075 Older Client Who Cannot Stay Asleep

Insomnia Disorder, Chronic · easy

Q2 · COUNSELING · R3

In session 1, Gloria lowers her voice to say she has not told anyone in her faith community about her sleep problems for fear of being seen as weak. How should the counselor respond at this moment?

was: What is the most appropriate response to her stigma about seeking help?

KEY **b.** Affirm her concern about appearing weak, normalize seeking help, and explore faith supports she already trusts.

near-miss **a.** Move directly into sleep hygiene education, assuming her stigma will fade once she notices symptom relief.

novice **c.** Challenge her belief that help-seeking signals weakness as a distortion needing cognitive restructuring.

harmful **d.** Recommend she limit weekly church involvement during treatment so religious beliefs do not slow her sleep progress.

☐ approve _____ re-tier _____ note

Q3 · COUNSELING · R2, R3

In session 2, Gloria says warmly that she is open to behavioral strategies and values approaches that honor her faith and community supports. How should the counselor structure the sleep-focused work?

was: Which approach best supports her engagement in sleep-focused work?

KEY **c.** Set sleep goals collaboratively, anchoring them to her faith values and existing community supports.

near-miss **a.** Deliver the standard CBT-I protocol as written, adding value-based tailoring only if adherence declines.

novice **d.** Focus sessions on exploring early-life roots of her fatigue before introducing any behavioral sleep tasks.

harmful **b.** Assign a fixed sleep schedule and warn that continued nonadherence will end the counseling relationship.

☐ approve _____ re-tier _____ note

Q4 · INTAKE · R1

As Gloria describes a year of broken sleep along with fatigue and irritability, which intake finding best supports chronic insomnia disorder rather than a depressive episode?

was: Which finding best supports chronic insomnia disorder over depression?

KEY **d.** Sleep difficulty occurs three or more nights weekly for a year despite adequate opportunity to sleep.

near-miss **a.** Daytime fatigue and irritability are present and interfere with her usual household functioning.

novice **b.** She denies suicidal ideation and appetite change, which rules out a co-occurring depressive disorder.

harmful **c.** Her low energy and irritability confirm a depressive disorder, making a separate insomnia diagnosis unnecessary.

☐ approve _____ re-tier _____ note

Q5 · TREATMENT · R2

Gloria has adequate sleep opportunity, no untreated comorbid disorder, and is open to behavioral work; which treatment should the counselor initiate first?

was: What is the recommended first-line treatment for chronic insomnia?

KEY **a.** Begin CBT-I with stimulus control and sleep restriction, tracked with a daily sleep diary.

near-miss **c.** Begin sleep hygiene education alone and reassess her sleep in eight weeks before adding other components.

novice **d.** Begin generalized anxiety worry-exposure work, treating her nighttime arousal as an anxiety disorder.

harmful **b.** Recommend nightly over-the-counter antihistamines for sleep and postpone behavioral treatment.

☐ approve _____ re-tier _____ note

Q6 · CORE · R1

When explaining the diagnosis to Gloria, which statement best captures the core features of her chronic insomnia disorder?

was: Which statement best captures the core features of her diagnosis?

KEY **b.** Dissatisfaction with sleep despite adequate opportunity, frequent and persistent, with daytime impairment.

near-miss **a.** Difficulty initiating and maintaining sleep on most nights over the past year, with adequate sleep opportunity.

novice **d.** Poor sleep arising from persistent worry and low mood, resolving once the underlying distress is treated.

harmful **c.** Reduced sleep expected with aging in older adults, requiring monitoring rather than a clinical diagnosis.

☐ approve _____ re-tier _____ note